Qualified Vendor Policy Manual

Policies and procedures for services delivered under the Arizona Department of Economic Security, Division of Developmental Disabilities (DDD) Qualified Vendor Agreement.

AGENCY	Foundation for Independence Holdings LLC
DBA	Foundation for Independence
CONTRACT #	QO7202618690
ADDRESS	8729 E. Hazel Street, Mesa, AZ 85208
APPROVED SERVICES	HAI/HID, GSE, ISE, ESA, PTE, TR1
VERSION	1.0 · October 27, 2026

Agency Profile

Who we are

Foundation for Independence Holdings LLC ("FFI," "the Agency," or "the Qualified Vendor") is an Arizona provider of person-centered Supported Living and Employment Services for adults and transition-age young adults with intellectual and developmental disabilities. FFI delivers services under a Qualified Vendor Agreement with the Arizona Department of Economic Security, Division of Developmental Disabilities (DES/DDD, "the Division").

Our mission is to help every member we serve build practical life skills, gain meaningful employment, and live with greater independence in their own community.

Agency information

Field	Detail
Legal name	Foundation for Independence Holdings LLC
Doing business as	Foundation for Independence (FFI)
Address	8729 E. Hazel Street, Mesa, AZ 85208
Phone (business and after-hours)	(602) 663-1664
Email	foundationforindependence@gmail.com
Website	https://www.ffiaz.info
DDD contract number	QO7202618690
OLCR certificate	LCR-237374-22582

Services covered by this manual

FFI is certified by the Office of Licensing, Certification and Regulation (OLCR) and contracted with the Division for the following services:

Service	DDD Code	Category
Habilitation, Supported Living (Hourly / Daily)	HAI/HID	Residential / In-Home Habilitation
Group Supported Employment	GSE	Employment Services
Individual Supported Employment	ISE	Employment Services

Service	DDD Code	Category
Employment Support Aide	ESA	Employment Services
Pathways to Employment	PTE	Employment Services
Transportation — Employment Related	TR1	Transportation Services

Organizational responsibility

Throughout this manual, responsibility for implementation, training, monitoring, and corrective action is assigned by title. FFI's leadership roles are:

Title	Core responsibilities
Executive Director	Overall policy ownership, contract compliance, final corrective action authority
Program Manager (Supported Living)	Day-to-day implementation and oversight for HAI/HID services
Employment Services Manager	Day-to-day implementation and oversight for GSE, ISE, ESA, PTE, and TR1
Quality & Compliance Coordinator	Policy tracking systems, audits, incident trending, quality management
Training Coordinator	Staff onboarding, training delivery, training records
On-Call Supervisor	After-hours coverage and emergency response at (602) 663-1664

In a small-agency configuration, one person may hold more than one title; accountability follows the title, not the individual.

How This Manual Is Organized

This manual was developed using the Division's **Policy Development Tool (DDD-2051A, 09/26)** and is organized so each FFI policy maps directly to the applicable DDD policy development checklist. The policy set reflects the services FFI is approved to provide (Supported Living and Employment Services, with employment-related Transportation); checklists that apply only to services FFI does not provide (for example, Developmental Homes, Attendant Care/Housekeeping monitoring, and Electronic Visit Verification) are intentionally not included.

Standard structure of every policy

In line with DDD-2051A 1.3 (Key Components for Written Policy Content), every policy in this manual contains:

1. **Document header** — document number, name, effective date, policy owner, and the services it applies to.
2. **Purpose and description** — why the policy exists and what it covers.
3. **Policy** — the standards FFI commits to.
4. **Procedures** — how staff carry the policy out, including required forms and timeframes.
5. **Training requirements** — what training supports the policy, who develops and delivers it, and how it is documented.
6. **Documentation and recordkeeping** — the records kept, where, and for how long.
7. **Monitoring and oversight** — the tracking systems used, the staff (by title) responsible for oversight, and how results feed continuous improvement.
8. **Reporting requirements** — internal and external reporting obligations and timeframes.
9. **Compliance and corrective action** — how infractions are addressed, from retraining through disciplinary action, and how systemic issues are corrected.
10. **Forms and references** — associated forms (appendix) and the DDD manuals, Arizona Administrative Code (A.A.C.), and Arizona Revised Statutes (A.R.S.) provisions the policy implements.

HCBS Rules commitment

FFI's policies are written to reflect full compliance with the federal Home and Community Based Services (HCBS) Rules issued by the Centers for Medicare and Medicaid Services (CMS). Members FFI serves have full access to the benefits of community living and receive services in the most integrated setting appropriate to their needs and preferences. See policy FFI-07 (HCBS Rules Compliance) for the agency-wide standard.

Definitions used throughout

Term	Meaning
Member	An individual determined eligible for and receiving Division-funded services

Term	Meaning
Responsible person	The member's guardian, designated representative, or (for minors) parent
DSP	Direct Support Professional — any FFI direct service staff
Planning Document / PCSP	The member's Person-Centered Service Plan and related planning documents
PRC	Program Review Committee
QVA	Qualified Vendor Agreement
Division / DDD	Arizona DES, Division of Developmental Disabilities

Review cycle

The full manual is reviewed at least annually by the Executive Director and Quality & Compliance Coordinator, and upon any change to the QVA, Division policy, or applicable law. Each policy's "Last Reviewed" date is updated when reviewed, whether or not changes were made.

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Part A — Governance & Compliance

FFI-01: HIPAA & Confidentiality

DOCUMENT NUMBER	FFI-01
POLICY OWNER	Executive Director
EFFECTIVE DATE	August 14, 2026
LAST REVIEWED	October 26, 2026
APPLIES TO	All FFI services (HAI/HID, GSE, ISE, ESA, PTE, TR1)
DDD PDT CHECKLIST	DDD-2051A § 2.14

1. Purpose

To protect the privacy and confidentiality of every member's records, protected health information (PHI), and personally identifiable information (PII) in compliance with HIPAA, A.R.S. § 36-568.01 and § 36-568.02, A.A.C. R6-6-808.G, R6-6-809.N, R6-6-810.A & B, R6-6-906.B, and DDD Medical Policy Manual Chapter 550 (Member Records and Confidentiality).

2. Policy

- FFI limits access to member records and PHI/PII to staff with a legitimate, service-related need to know.
- No member information is released to any outside party without a signed, informed consent for release of information from the member or responsible person, except where disclosure is required by law (for example, mandatory reporting, Division requests, or court order).
- Members' confidential information is safeguarded every day, in every setting — in the office, in members' homes, at worksites, in vehicles, and online.
- Confidentiality breaches are incidents. They are reported verbally and in writing under FFI-02 (Incident Reporting), consistent with DES/DDD incident reporting procedure.

3. Procedures

3.1 Safeguards and storage

- Physical records are stored in locked cabinets at the FFI office; keys are limited to the Executive Director, Program Manager, and Quality & Compliance Coordinator.
- Electronic records are stored in access-controlled, encrypted systems with unique logins per staff member; access rights follow each staff member's role and are revoked on separation.
- Records in daily use (for example, service notes at a member's home or worksite) are kept out of view of visitors and returned to secure storage the same day.
- Daily safeguards include: screens locked when unattended, no member documents left in vehicles, no discussion of member information in public areas, and secure (encrypted) email for any transmission of PHI.

3.2 Sharing member information

- Before sharing any member information, staff verify and document: who is asking, what specific information is requested, the purpose, and whether a current signed release covers the request.
- Requests are logged by the Quality & Compliance Coordinator in the member's record.
- The Division, and entities authorized by law, may access records upon request.

3.3 Informed consent and photography

- A signed Release of Information form is obtained before any disclosure of confidential information and renewed at least annually or when the scope changes.
- A separate, specific Photography/Media Consent is required before any photo, video, or audio of a member is taken, used, or displayed. Consent may be limited or withdrawn at any time.

3.4 Social media, marketing, and advertising

- Staff must not post, share, or reference any member (name, image, identifiable detail, or service relationship) on any social networking site, personal or agency, without a current Photography/Media Consent and Executive Director approval.
- FFI marketing and advertising materials never include member PHI or identifiable member information without documented consent.

3.5 Portable equipment

- Only FFI-approved devices (agency-issued or approved personal smartphones, laptops, and encrypted flash drives) may hold or access member information.
- Devices must be password-protected and encrypted; member information may not be stored on personal cloud accounts. Lost or stolen devices are reported immediately to the Executive Director and treated as a potential breach.

3.6 Records destruction

- Records past their retention period (see FFI-05) are destroyed by cross-cut shredding (paper) or certified secure deletion (electronic). Destruction is logged with date, records description, and the staff member responsible.

4. Training requirements

- HIPAA/Confidentiality training is completed at hire (before working with members) and annually thereafter, delivered in person or online by the Training Coordinator, and includes review of A.R.S. § 36-568.01.
- Training covers safeguards, consent procedures, photography/social media rules, portable equipment, and breach reporting. Comprehension is verified by a written or electronic quiz; results are kept in the personnel file.

5. Documentation and recordkeeping

Signed confidentiality agreements, consents, release logs, breach reports, and destruction logs are maintained per FFI-05 (minimum seven years).

6. Monitoring and oversight

The Quality & Compliance Coordinator audits a sample of member files and staff practices quarterly (storage, consents on file, release logs) and reviews findings with the Executive Director. This policy is reviewed with all staff and members at least annually and when revised.

7. Compliance and corrective action

Violations result in corrective action proportionate to the breach — from retraining, up to termination and referral to authorities where required. Any breach affecting a member is reported to the Division per FFI-02, with an internal review to correct the system weakness that allowed it.

8. Forms and references

Forms: FFI Release of Information; FFI Photography/Media Consent; FFI Confidentiality Agreement (staff); AHCCCS QM Portal IAD report (FFI-02) for breaches that are reportable incidents.

References: QVA Standard Terms and Conditions; DDD Medical Policy Manual Ch. 550; A.R.S. § 36-568.01, § 36-568.02; A.A.C. R6-6-808.G, R6-6-809.N, R6-6-810.A & B, R6-6-906.B; HIPAA Privacy and Security Rules (45 CFR Parts 160, 164).

FFI-02: Incident Reporting

DOCUMENT NUMBER	FFI-02
POLICY OWNER	Executive Director
EFFECTIVE DATE	August 14, 2026
LAST REVIEWED	October 26, 2026
APPLIES TO	All FFI services (HAI/HID, GSE, ISE, ESA, PTE, TR1)
DDD PDT CHECKLIST	DDD-2051A § 2.15

1. Purpose

To ensure every incident affecting a member's health, safety, or rights is recognized, responded to, documented, and reported consistent with DES/DDD policy (Provider Policy Manual Chapter 70; Operations Policy Manual Chapters 2000, 6000, 6002; Behavior Supports Manual Chapter 900; A.A.C. R6-6-803, R6-6-807, R6-6-809, R6-6-901).

2. What constitutes an incident

FFI uses the Division's definitions (Operations Manual 6002A) and AHCCCS Medical Policy Manual (AMPM) Policy 961. Examples include, but are not limited to:

Category	Examples
Sentinel events	Death; alleged abuse, neglect, or exploitation; missing member; suicide attempt; serious injury requiring medical treatment; law enforcement involvement; medication errors with harm; emergency measures/physical management
Reportable incidents	Minor injury; medication errors without harm; property damage; behavioral incidents without harm; service delivery disruptions

Member health and safety always comes first: staff stabilize the situation, render aid, and call 911 when needed **before** any documentation.

3. Emergency roles and responsibilities

- **Medical emergency (e.g., choking):** the DSP on duty immediately performs trained first aid/CPR/abdominal thrusts as applicable, calls 911, does not leave the member alone, and notifies the On-Call Supervisor once the member is safe.
- **Fire or flood:** the DSP evacuates the member(s) to the designated safe location, calls 911, and follows the site evacuation plan (FFI-26).
- Emergency numbers, after-hours contacts for FFI management (On-Call Supervisor: (602) 663-1664), and emergency responder numbers are posted at each service site and carried in each service vehicle.

4. Reporting procedures and timeframes

1. **Immediate verbal report.** For Sentinel events, staff verbally notify the On-Call Supervisor immediately, and the Executive Director notifies the Division immediately (using the Division's after-hours line, (602) 375-1403 or 1-855-375-1403, outside business hours). Allegations of abuse, neglect, or exploitation additionally trigger mandatory reports to APS (1-877-767-2385), DCS (1-888-767-2445), and/or law enforcement or Tribal Social Services as applicable, before the end of the shift.
2. **Entry into the AHCCCS QM Portal.** All reportable incidents are entered directly into the AHCCCS Quality Management (QM) Portal as an Incident, Accident, and Death (IAD) report, following the Division's AHCCCS QM Portal Migration User Guide and FAQs and the Provider Quick Guide for Incident Reporting.
    - **Sentinel events:** reported to the Division immediately, and entered into the QM Portal within one business day of the event.
    - **Reportable incidents:** entered into the QM Portal within two business days of the incident or of FFI being notified of it.
3. **Minimum required information.** Every QM Portal entry includes, at a minimum, the information required by the portal's IAD steps:
    - the member (name, date of birth, AHCCCS ID) and member information;
    - provider information (FFI, the service site, and the assigned Clinical Director);
    - basic incident information: date, time, location, and incident category/subcategory;
    - an objective, chronological description of the incident;
    - the member's condition before and after the incident, and any medical services received;
    - witnesses, including staff present;
    - provider actions taken and actions recommended to prevent recurrence;
    - notifications made (guardian/responsible person, Support Coordinator, APS/DCS, law enforcement), with names, dates, and times; and
    - supporting attachments, when relevant (for example, staff statements).
4. **Notifications.** The Program Manager or Employment Services Manager notifies the member's guardian/responsible person, DDD Support Coordinator, and (when involved in the member's care) Behavioral Health providers, the same day for Sentinel events and within one business day otherwise, and documents who was notified and when.
5. **QM Portal user roles.** The Executive Director is FFI's designated point of contact for all QM and incident reporting inquiries and holds FFI's QM Portal account. Responsibilities follow the portal's user roles:
    - **Preparer** — creates the IAD report from the staff member's account of the incident and e-signs it. The Executive Director is the Preparer until additional staff are granted the role; when they are, the Program Manager or Employment Services Manager prepares reports for their programs.
    - **Clinical Director** — reviews each report for accuracy and completeness, then approves it or returns it to the Preparer for correction. The Executive Director holds this role; when another person holds the Preparer role, the Clinical Director is never the same person who prepared the report.

- FFI registers its QM Portal account within 30 days of becoming an AHCCCS-registered provider. Until the account is active, the Executive Director contacts Provider Network Support (providernetworksupport@azdes.gov) and reports incidents using the Division's current instructions, so that no reporting deadline is missed.

6. **Administrative review and event status.** Every report is reviewed for accuracy and completeness before final submission. Any revision is verified by the staff member who wrote the original account, and the original account is attached to the final report sent to the Division. The Preparer and Clinical Director track each report through its QM Portal status until it is submitted to the Division:
   - **Draft** — the Preparer saves and completes all required steps, then confirms the Report Validation step shows no missing information;
   - **Signed by the Preparer** — the Preparer e-signs, which sends the report to the assigned Clinical Director;
   - **Returned** — the Clinical Director returns a report that needs correction; the Preparer corrects it and e-signs again the same business day;
   - **Approved** — the Clinical Director approves and e-signs the report, which submits it to the Division for review; the Executive Director confirms the submission, and responds to any Division follow-up questions within the timeframe requested; and
   - **Withdrawn** — used only when the Clinical Director determines an entry is not reportable or duplicates a report already submitted, with the reason documented.

   The Executive Director checks open QM Portal reports every business day so that no report remains in Draft or Returned status past the deadlines above.

5. Incidents involving staff

If an alleged incident involves paid staff — including any allegation of abuse, neglect, or exploitation — the staff member is immediately removed from work with or contact with members pending the outcome of the investigation/fact-finding. Reassignment or leave is determined by the Executive Director.

6. Fact-finding

The Quality & Compliance Coordinator (or Executive Director when the Coordinator is involved) conducts internal fact-finding: securing the scene/records, interviewing staff and witnesses, reviewing documentation, and producing a written summary with corrective actions. FFI cooperates fully with Division, APS, DCS, and law enforcement investigations.

7. Training requirements

All staff complete incident reporting and mandatory reporting training at hire (before working with members) and annually, delivered by the Training Coordinator (in person or online), including how incidents are entered in the AHCCCS QM Portal, differentiating Sentinel events from reportable incidents, and the Provider Quick Guide for Incident Reporting. Staff sign an acknowledgment that they understand they are mandatory reporters. This policy is reviewed with all direct care staff at least annually and whenever revised, with signed verification.

8. Documentation, monitoring, and oversight

- All incident reports, fact-finding records, and notification logs are retained per FFI-05.
- The Quality & Compliance Coordinator logs every incident in a tracking system (date, member, type, severity, notifications, corrective actions, closure date) and reviews trends quarterly with the Executive Director; trends feed the Quality Management Plan (FFI-06).

9. Compliance and corrective action

Failure to report, late reporting, or falsification results in disciplinary action up to termination and any legally required referral. Systemic gaps identified through trending result in corrective action plans with assigned owners and completion dates.

10. Forms and references

Forms: AHCCCS QM Portal IAD report; Provider Quick Guide for Incident Reporting; AHCCCS QM Portal Migration User Guide and FAQs; DDD Incident Report (DDD-0191A), used only when the Division directs it; FFI notification log.

References: QVA Standard Terms and Conditions and Service Requirements; DDD Operations Policy Manual Ch. 2000, 6000, 6002; Behavior Supports Manual Ch. 900; Provider Policy Manual Ch. 70; AMPM Policy 961; A.A.C. R6-6-803, R6-6-807, R6-6-809, R6-6-901; incorporated policies FFI-01 (HIPAA), FFI-03 (Neglect & Abuse), FFI-04 (FWA).

FFI-03: Neglect, Abuse & Exploitation Prevention

DOCUMENT NUMBER	FFI-03
POLICY OWNER	Executive Director
EFFECTIVE DATE	August 14, 2026
LAST REVIEWED	October 26, 2026
APPLIES TO	All FFI services (HAI/HID, GSE, ISE, ESA, PTE, TR1)
DDD PDT CHECKLIST	DDD-2051A § 2.26

1. Purpose

To define, prevent, detect, and respond to neglect, abuse, and exploitation of members, consistent with DDD Provider Policy Manual Chapters 20 and 64, Operations Policy Manual Chapters 2000, 6000, and 6002-G, A.A.C. R6-6-1600, R6-6-808.I, R6-6-809.C.2 & D, R6-6-906.B, and A.R.S. §§ 36-551.01, 36-569, 13-3620, and 46-454.

2. Definitions

- **Abuse:** intentional infliction of physical, emotional, or sexual harm — including hitting, rough handling, intimidation, humiliation, verbal assault, or any sexual contact with a member.
- **Neglect:** failure to provide the supervision, food, medical care, or services a member needs, resulting in or risking harm.
- **Exploitation:** use of a member or a member's resources (money, property, labor, image) for another person's benefit.

3. Policy

- FFI has zero tolerance for abuse, neglect, or exploitation by anyone — staff, family, community members, or other members.
- **All FFI staff are mandatory reporters** under A.R.S. § 13-3620 (children) and § 46-454 (vulnerable adults). No supervisor approval is needed — or permitted to be required — before a staff member makes a mandated report.
- The DES/DDD-approved sign, **"Everyone Has the Right to be Safe,"** is posted at every service setting's telephone location and/or next to posted emergency numbers.
- The following behavioral intervention techniques are **prohibited** in all FFI settings: locked time-out rooms; overcorrection; application of noxious stimuli; and physical, mechanical, or pharmacologic restraints used as a negative consequence to behavior.
- Misuse or abuse of medications (routine, PRN, or one-time — including using medication to sedate or punish) is abuse and is reported and investigated as such.

4. Recognizing abuse and neglect

Staff are trained to recognize indicators, including: unexplained bruises, burns, or injuries; fear or withdrawal around specific people; sudden behavior changes; poor hygiene or weight loss; missing funds or property; isolation; and inconsistent explanations for injuries. Detection duty extends to suspected abuse **occurring outside FFI services** (home, community, other providers): staff report what they observe regardless of where it may have occurred.

5. Response procedures

1. **Protect first.** Immediately separate the member from the suspected source of harm, render aid, and call 911 if there is injury or danger.
2. **Report.** Verbally report immediately to APS (1-877-SOS-ADULT) for adults or DCS (1-888-SOS-CHILD) for children, law enforcement and/or Tribal Protective Services as applicable, and the Division; notify the On-Call Supervisor. The incident is entered into the AHCCCS QM Portal within the timeframes in FFI-02.
3. **Remove implicated staff.** Any staff member alleged to have abused, neglected, or exploited a member is removed from member contact immediately pending investigation outcome.
4. **Immediate interventions.** The Program Manager/Employment Services Manager puts protective measures in place the same day (staffing changes, increased supervision, environmental fixes) to prevent further harm.
5. **Review and corrective action.** The Quality & Compliance Coordinator completes fact-finding, cooperates with external investigations, and develops a written corrective action plan with owners and due dates; the Executive Director approves and monitors it to completion.

6. Intervention techniques and treatment considerations

Support strategies must follow the member's Planning Document and Behavior Treatment Plan (FFI-15). When interventions, treatments, or medications carry risks or side effects that may adversely affect a member, those risks are documented, disclosed to the member/responsible person, and monitored by the planning team.

7. Member training

FFI reviews personal-safety information with members at intake and at least annually, in plain language and the member's preferred communication mode: what abuse/neglect/exploitation are, that they have the right to be safe, and how to tell someone (including FFI's number, APS, and the "Everyone Has the Right to be Safe" poster).

8. Staff training

Abuse/neglect/exploitation prevention training (including mandatory reporting, recognition, prohibited techniques, and this policy) is completed at hire before member contact and annually thereafter, delivered by the Training Coordinator; competency is verified and documented with signed acknowledgment.

9. Monitoring, compliance, and corrective action

The Quality & Compliance Coordinator tracks all allegations, reports, investigation outcomes, and corrective action plans, and reviews trends quarterly with the Executive Director (fed into FFI-06). Violations of this policy result in

disciplinary action up to termination, mandatory referral to authorities, and system-level corrective action plans (CAPs) for any process failure.

10. Forms and references

Forms: AHCCCS QM Portal IAD report (FFI-02); "Everyone Has the Right to be Safe" poster; FFI corrective action plan template.

References: QVA Standard Terms and Conditions; DDD Operations Policy Manual Ch. 2000, 6000, 6002-G; Provider Policy Manual Ch. 20, 64; A.A.C. R6-6-1600, R6-6-808.I, R6-6-809.C.2 & D, R6-6-906.B; A.R.S. §§ 36-551.01, 36-569, 13-3620, 46-454.

FFI-04: Fraud, Waste & Abuse / False Claims

DOCUMENT NUMBER	FFI-04
POLICY OWNER	Executive Director
EFFECTIVE DATE	August 14, 2026
LAST REVIEWED	October 26, 2026
APPLIES TO	All FFI services (HAI/HID, GSE, ISE, ESA, PTE, TR1)
DDD PDT CHECKLIST	DDD-2051A § 2.12

1. Purpose

To prevent, detect, and report fraud, waste, and abuse (FWA) in Division-funded services, consistent with DDD Provider Policy Manual Chapters 20 (Fraud, Waste and Abuse) and 21 (False Claims Act).

2. Prevention and detection

- FFI bills only for services actually delivered, by qualified staff, to authorized members, in authorized amounts, supported by contemporaneous documentation.
- Internal controls include: separation of duties between service documentation and billing; supervisor verification of timesheets against service logs (FFI-05); monthly reconciliation of claims against authorizations; monthly SAM and OIG/LEIE exclusion checks for all staff; and random documentation audits by the Quality & Compliance Coordinator.

3. Code of Conduct and misconduct

FFI maintains a well-publicized Code of Conduct, distributed at hire and posted at the office, requiring honesty in documentation and billing and prohibiting falsification of any record.

- **Reporting suspicious actions:** any suspicious action observed by or reported to staff — by a member, representative, or another staff member — must be reported to the Executive Director or Quality & Compliance Coordinator immediately. Reports may be made anonymously.
- **Referral and reporting:** confirmed or reasonably suspected FWA is reported to the Division (DDD Customer Service / Office of Inspector General referral channels) and, where applicable, AHCCCS OIG. FFI cooperates fully with all investigations.
- **Whistleblower protections:** FFI prohibits retaliation of any kind against anyone who reports suspected FWA in good faith, consistent with federal and state whistleblower protections (including FCA qui tam provisions, 31 U.S.C. § 3730(h), and A.R.S. § 23-1501).
- **Investigative process:** the Executive Director assigns an investigator (normally the Quality & Compliance Coordinator), who secures records, interviews witnesses, documents findings, and recommends corrective action

within 30 days; findings involving payments trigger repayment and self-disclosure as required.

4. False Claims Act and related laws

Staff are trained on the following, which apply to all federally funded programs:

- **False Claims Act (31 U.S.C. §§ 3729–3733):** it is a violation to knowingly submit a false or fraudulent claim for payment; to make or use a false record or statement to conceal, avoid, or decrease an obligation to pay or transmit money or property to the Federal Government; or to conspire with others to have a false or fraudulent claim paid by the Federal Government. The FCA covers fraud involving any federally funded contract or program, with the exception of tax fraud. Remedies include treble damages and per-claim civil penalties; qui tam whistleblowers may share in recoveries and are protected from retaliation.
- **Deficit Reduction Act of 2005:** requires entities receiving significant Medicaid funds to educate employees about the FCA, state false claims laws, whistleblower protections, and the entity's own FWA policies — this policy and its training satisfy that requirement.
- **Stark Law (42 U.S.C. § 1395nn):** prohibits physician self-referral for designated health services where a financial relationship exists.
- **Anti-Kickback Statute (42 U.S.C. § 1320a-7b):** prohibits knowingly offering, paying, soliciting, or receiving anything of value to induce or reward referrals of federal health care program business.
- **Arizona law:** A.R.S. § 36-2918 (AHCCCS false claims civil penalties) and A.R.S. § 13-2310 (fraudulent schemes, a class 2 felony) provide state criminal and civil penalties for false claims and statements.

5. Training requirements

FWA/FCA training is completed at hire and annually for all staff (delivered by the Training Coordinator, in person or online), covering this policy, the Code of Conduct, the laws above, how to report, and whistleblower protections. Completion and comprehension are documented.

6. Monitoring and oversight

The Quality & Compliance Coordinator owns the FWA monitoring calendar (exclusion checks monthly; billing reconciliation monthly; documentation audits quarterly) and reports results to the Executive Director. The Program Integrity Requirements Verification (DDD-2090A) is maintained current.

7. Compliance and corrective action

Substantiated FWA results in disciplinary action up to termination, repayment, disclosure to the Division/AHCCCS, and referral for prosecution where warranted. Control weaknesses identified are corrected through written CAPs tracked to completion.

8. Forms and references

Forms: FFI Code of Conduct; DDD-2090A Program Integrity Requirements Verification; FFI FWA report log.

References: DDD Provider Policy Manual Ch. 20, 21; False Claims Act; Deficit Reduction Act of 2005; Stark Law; Anti-Kickback Statute; A.R.S. § 36-2918, § 13-2310.

FFI-05: Record Retention, Data Security & Timesheet Verification

DOCUMENT NUMBER	FFI-05
POLICY OWNER	Quality & Compliance Coordinator
EFFECTIVE DATE	August 14, 2026
LAST REVIEWED	October 26, 2026
APPLIES TO	All FFI services (HAI/HID, GSE, ISE, ESA, PTE, TR1)
DDD PDT CHECKLIST	DDD-2051A § 2.28

1. Purpose

To establish consistent recordkeeping, retention, data security, and time verification standards across all FFI settings, per the QVA (Standard Terms and Conditions 6.3.1 — current, up-to-date employee and member records at all times), DDD Provider Policy Manual Chapter 35 (Progress Reporting), and A.A.C. R6-6-809.N.

2. Documentation requirements

- **Employee hours:** all employee hours are tracked and verified. DSP time entries identify the member served, service code, date, start/end times, and location.
- **Verification of hours:** the member or guardian signs off on/verifies hours worked by the employee in writing (paper or electronic signature) on a recurring basis; supervisors reconcile timesheets against schedules and service notes before payroll and billing.
- **Proof of hours:** timesheets, verification signatures, and payroll records are retained as proof of hours worked.
- **Attendance logs:** where applicable (group settings, PTE/GSE program activities), FFI maintains a log of each member's attendance including arrival and departure times.
- **Teaching strategies:** habilitation teaching strategies (DDD-1783A format) are developed and submitted within 20 business days of initiating service, of a change, or when a new outcome is identified.

3. Progress reporting

Service	Report	Deadline
HAI/HID (Supported Living) and other non-group-home habilitation	Quarterly progress report (DDD-1784A format) to DDD, member/representative, and PCP	July 15, October 15, January 15, April 15
PTE, ISE, GSE, ESA	Quarterly progress report to DDD, member/representative, and PCP	July 15, October 15, January 15, April 15
PTE, ISE, GSE, ESA	Semi-annual report to District Program Manager or designee	January 31 and July 31

Progress reports are submitted through the Division's secure FTP process; reports are written in the language of the member/responsible person (interpretation/translation funded by FFI, see FFI-13).

4. Retention

All member records, service documentation, employee records, timesheets, ledgers, incident reports, and billing records are retained a minimum of **seven (7) years** (longer where a specific law requires), then destroyed securely per FFI-01 § 3.6.

5. Data security

- Electronic systems: unique user accounts, role-based access, strong passwords, encryption at rest and in transit, and prompt deactivation on staff separation.
- Paper records: locked storage, sign-out procedure for temporary removal, no records left unattended off-site.
- Backups of electronic records are maintained and tested; loss of systems/records is covered by the Contingency Plan (FFI-26).

6. Publications review

Any report or publication FFI intends for DDD members or applicants for Division-funded services is provided to DDD for review before distribution.

7. Training, monitoring, and oversight

The Training Coordinator trains all staff at hire and annually on documentation standards and timesheet integrity. The Quality & Compliance Coordinator audits timesheet/service-note reconciliation monthly, spot-checks member files quarterly, tracks progress report timeliness against the calendar above, and reports compliance to the Executive Director.

8. Compliance and corrective action

Falsified time or service records are treated under FFI-04 (FWA) and result in discipline up to termination and repayment. Missed report deadlines trigger a documented cause review and CAP.

9. Forms and references

Forms: FFI timesheet with member/guardian verification; attendance log; DDD-1783A Habilitation Outcomes Strategies; DDD-1784A Habilitation Progress Report; ISE/GSE/ESA/PTE quarterly and six-month report formats.

References: QVA Standard Terms and Conditions & Service Requirements (Recordkeeping sections of all Service Specifications); DDD Provider Policy Manual Ch. 35, 41; A.A.C. R6-6-809.N.

FFI-06: Quality Management Plan

DOCUMENT NUMBER	FFI-06
POLICY OWNER	Executive Director
EFFECTIVE DATE	August 14, 2026
LAST REVIEWED	October 26, 2026
APPLIES TO	All FFI services (HAI/HID, GSE, ISE, ESA, PTE, TR1)
DDD PDT CHECKLIST	DDD-2051A § 2.27

1. Purpose

To define FFI's Quality Management (QM) program: how incidents, complaints, member/family input, service monitoring, and improvement activities are managed and used to continuously improve services, per the QVA Service Requirements, Provider Policy Manual Chapters 32 and 42, Behavior Supports Manual Chapter 900, and A.A.C. R6-6-901.

2. Incident management (summary — see FFI-02)

- Incidents are reported internally (staff → On-Call Supervisor → management) and externally (Division, APS/DCS/law enforcement as applicable) within the timeframes in FFI-02.
- Family/guardian, Division, and (as applicable) police/APS/DCS notifications follow FFI-02 timelines (same day for serious incidents; next business day otherwise).
- Reporting protocols are shared with members, families, and guardians at intake and annually.
- Every incident receives an internal review; corrective actions are assigned, implemented, and monitored to completion by the Quality & Compliance Coordinator.
- Incidents are logged and coded for tracking and trending; trends are reviewed quarterly.

3. Complaints and grievances

- **Who can file:** any member, family member, guardian/representative, staff member, or other stakeholder, verbally or in writing, including anonymously.
- **Who resolves:** the Program Manager or Employment Services Manager resolves service-level complaints; the Executive Director resolves complaints about managers or the agency.
- **Timeframe:** acknowledgment within 2 business days; resolution or written status within 10 business days; complex matters no later than 30 days.
- **Monitoring:** the Quality & Compliance Coordinator logs all complaints/grievances, monitors resolution timeliness, and trends them quarterly alongside incidents.

4. Member, family, and representative input

- Input is actively solicited through: the annual Family Feedback/Satisfaction Survey (FFI-14), planning meetings, and an open-door channel to management.
- Satisfaction is measured through the annual survey plus complaint/compliment tracking.
- Members/families are invited to participate in the evaluation of direct service staff (feedback is included in staff performance reviews) and in the evaluation and improvement of services.
- FFI commits to at least these activities for active involvement in operations:
    1. **Periodic member/family meetings and open houses** to share agency updates and solicit feedback on policies, procedures, and program enhancements; and
    2. **Sharing improvement activity results** (survey results, QM outcomes) with members, families, and representatives at least annually.
- Members and families are encouraged to self-direct and manage their services to the extent they wish, consistent with person-centered planning.

5. Monitoring and evaluation of services

- **Service gap analysis:** the Quality & Compliance Coordinator compares authorized versus delivered hours monthly, identifies gaps, reports service gaps to the Division, and drives staffing fixes (FFI-19/FFI-20).
- **Planning Document alignment:** supervisors review service notes against each member's Planning Document objectives monthly; progress reports (FFI-05) formalize this quarterly.
- **Member well-being:** additional systematic monitoring includes supervisor site visits, observation of DSP-member interactions, and health status review (FFI-16).
- **Training quality:** training effectiveness is evaluated through post-training assessments, in-class feedback, and supervisor drop-in observations of practice.
- **Financial integrity:** billing accuracy and timeliness are ensured through the controls in FFI-04 and FFI-05.
- **Plan governance:** this QM Plan is developed by the Executive Director with the Quality & Compliance Coordinator, shared with all staff and with families on request, and reviewed and updated at least annually.
- **Physical environments:** FFI-operated or leased environments are evaluated quarterly for cleanliness, safety, and risk factors using a site checklist; issues are corrected on a tracked CAP.

6. Electronic monitoring/surveillance (employment, day program, and residential settings)

If FFI installs or uses any electronic monitoring or surveillance equipment:

1. FFI notifies the District Network Manager **before** installation or use and provides policies/procedures/notices demonstrating no member rights under A.R.S. § 36-551.01 are violated.
2. In residential settings, surveillance is used only where residing members and their legal representatives request or consent to it.
3. Surveillance in common public settings (e.g., workshops, employment programs) is permitted.

4. A sign is posted conspicuously in each common area under surveillance, indicating days and hours of surveillance.
5. Surveillance never extends to members' private spaces (bathrooms, bedrooms).
6. Surveillance records are retained per A.R.S. § 12-2297 and produced on request to the member or responsible person, the Division, law enforcement, protective agencies, and others entitled to access.

7. Emergency safeguards and improvement projects

Safeguards protecting members in natural disasters or public emergencies are defined in FFI-26. When FFI defines annual Quality Improvement projects, each has a written aim, measures, and owner; results are analyzed against the aim and shared with staff, members, and families.

8. Oversight, compliance, and corrective action

The Executive Director holds overall QM accountability; the Quality & Compliance Coordinator operates the tracking systems and quarterly QM review. Failures to follow QM processes result in retraining and progressive discipline; systemic issues result in documented CAPs tracked to completion.

9. Forms and references

Forms: FFI complaint/grievance log; incident tracking log; site safety checklist; annual survey (FFI-14); CAP template.

References: QVA Service Requirements/Scope of Work; Provider Policy Manual Ch. 32, 42; Behavior Supports Manual Ch. 900; A.A.C. R6-6-901; A.R.S. § 36-551.01, § 12-2297.

FFI-07: HCBS Rules Compliance

DOCUMENT NUMBER	FFI-07
POLICY OWNER	Executive Director
EFFECTIVE DATE	August 14, 2026
LAST REVIEWED	October 26, 2026
APPLIES TO	All FFI services (HAI/HID, GSE, ISE, ESA, PTE, TR1)
DDD PDT CHECKLIST	DDD-2051A § 1.3.1 / 1.5 (HCBS Rules row)

1. Purpose

To ensure FFI's services fully comply with the CMS Home and Community Based Services (HCBS) Final Rule (Medicaid HCBS Final Regulation; AHCCCS HCBS Rules), whose purpose is to ensure individuals receiving HCBS are integrated into their communities and have full access to the benefits of community living, receiving services in the most integrated setting.

2. Policy

FFI applies the guiding principles of the ALTCS program — member-centered case management, person-centered planning that safeguards against unjustified restrictions, and stakeholder collaboration — across all services. For every member, FFI ensures:

1. **Community integration.** Services support full access to the greater community: community activities, competitive integrated employment (the explicit aim of ISE/GSE/PTE/ESA), and control of personal resources.
2. **Choice.** Members choose their services, providers, activities, schedules, and (in supported living) where and with whom they live, from among setting options including non-disability settings.
3. **Rights.** Members' rights of privacy, dignity, respect, and freedom from coercion and restraint are protected in every setting; FFI's prohibited-techniques rules (FFI-03) apply everywhere.
4. **Autonomy.** Services optimize individual initiative, autonomy, and independence in making life choices, including daily activities, physical environment, and with whom to interact.
5. **No unjustified restrictions.** Any modification of these conditions must be supported by a specific assessed need, justified and documented in the person-centered Planning Document with informed consent, tried-and-failed less intrusive alternatives, data collection, and review dates — never for staff convenience.
6. **Supported living settings (HAI/HID).** Members' homes are the member's own; members control their schedules, have access to food at any time, may have visitors of their choosing at any time (see FFI-22), and hold the same responsibilities and protections from eviction as any tenant under Arizona landlord-tenant law.

3. Procedures

- Person-centered planning participation per FFI-11; FFI never restricts a member except through the documented Planning Document process above.
- Staff raise potential rights restrictions to the Program Manager, who brings them to the member's planning team before any restriction is implemented.
- Annual self-assessment: the Quality & Compliance Coordinator completes an HCBS settings self-assessment for each service line annually and remediates any gaps by CAP.

4. Training

HCBS Rules training (community integration, member rights, person-centered practice, prohibition on unjustified restrictions) is included in onboarding and annual refreshers, delivered by the Training Coordinator; competency is documented.

5. Monitoring, compliance, and corrective action

The Quality & Compliance Coordinator monitors HCBS compliance through the annual self-assessment, site visits, planning document reviews, and member/family feedback. Violations (unjustified restriction, rights interference) are corrected immediately, reported where required, and addressed through retraining and discipline.

6. Forms and references

Forms: FFI HCBS settings self-assessment; Planning Document restriction-justification record.

References: Medicaid HCBS Final Regulation (42 CFR 441.301(c)(4)-(5)); AHCCCS Home and Community-Based Services rules; DDD Provider Policy Manual Ch. 61; A.R.S. § 36-551.01.



Part B — Personnel & Training

FFI-08: Personnel, Hiring & Training Plan

DOCUMENT NUMBER	FFI-08
POLICY OWNER	Executive Director
EFFECTIVE DATE	August 14, 2026
LAST REVIEWED	October 26, 2026
APPLIES TO	All FFI services (HAI/HID, GSE, ISE, ESA, PTE, TR1)
DDD PDT CHECKLIST	DDD-2051A § 2.23

1. Purpose

To define the qualifications, background screening, and training every FFI staff member must meet before and while working with members, per the QVA, DDD Provider Policy Manual Chapters 20, 26, 49, and 61, A.A.C. R6-6-808, R6-6-809.C, R6-6-905, R6-6-906, and A.R.S. §§ 36-558.01 and 36-593.01.

2. Hiring requirements

Before working with members, every direct service staff member must have on file:

1. **Valid Class 1 Fingerprint Clearance Card** (verified via the AZ DPS website; front and back copied; validity re-verified at renewal).
2. **Minimum age of 18 years.**
3. **Notarized attestation** that the person is not: (i) subject to registration as a sex offender in Arizona or any other jurisdiction; (ii) awaiting trial on, or convicted of, committing or attempting, soliciting, facilitating, or conspiring to commit any criminal offense listed in A.R.S. § 41-1758.03(B); or (iii) any similar offense in another state or jurisdiction.
4. **Notarized Criminal History Self-Disclosure Declaration Statement (LCR-1034A)** — renewed at least every 3 years; if signed annually, it may serve in place of the attestation above.
5. **Direct Service Position Declaration Form (DDD-1727A).**
6. **Three references**, documented and available for DDD verification.
7. **CPR certification** — renewed every 2 years (in-person skills demonstration).
8. **First Aid training/certification** — renewed every 2 years.
9. **Monthly SAM and OIG/LEIE exclusion checks** (ongoing, all staff).
10. **Central Registry background checks** (DCS and APS Registry).
11. **Article 9 review** with written documentation — renewed every 3 years, taught by a certified instructor.
12. **Any additional training required by an assigned member's Planning Document** before serving that member.

13. **P.R.O.U.D. (Providing Respectful Opportunities and Understanding De-escalation)**, if applicable: required before working with a member who has a behavior plan or documented unsafe behavior. DSPs hired after July 31, 2026 complete P.R.O.U.D.; staff with current Prevention & Support certification complete P.R.O.U.D. by December 31, 2027 (FFI-15 §2).
14. **Verification of date of hire, AZ driver's license (if driving), I-9 / E-Verify lawful presence, and previous experience.**
15. **Direct Care Worker (DCW) Training, as applicable:** when a service FFI provides requires AHCCCS DCW training and testing, the staff member completes it before providing that service, and completion is recorded in the training matrix (§3.3).

3. Training plan

3.1 Core onboarding (before member contact)

Orientation to FFI mission and values; member rights (Article 9); HIPAA/confidentiality (FFI-01); incident reporting and mandatory reporting (FFI-02, FFI-03); FWA (FFI-04); person-centered practice and the PCSP process (FFI-11); cultural competency (FFI-13); health and medical response (FFI-16); documentation standards (FFI-05); pre-service orientation to each assigned member (FFI-09).

3.2 Service-specific training

- **Supported Living (HAI/HID):** habilitation outcomes and teaching strategies (DDD-1783A), daily-living skill building, community integration, and any member-specific health/behavior protocols.
- **Employment services (GSE/ISE/ESA/PTE):** job coaching methods, employer engagement, natural supports, benefits basics, and worksite safety (including Arizona OSHA awareness).
- **Transportation (TR1):** responsible driving (Provider Manual Ch. 49) and FFI-27 requirements.

3.3 Tracking system

The Training Coordinator maintains a training and qualification matrix for every employee (dates completed, expiration dates for CPR/First Aid/FPC/Article 9/P.R.O.U.D., member-specific trainings) with automated expiry reminders at 60/30/7 days. FFI's license/training tracking system is available to the Division on request.

4. Key requirements (verification and enforcement)

1. Staff violations of policy are covered in this manual's compliance sections and FFI's disciplinary procedure.
2. Every policy in this manual identifies its supporting training.
3. Training frequency is defined per topic (onboarding, annual refresher, 2-year CPR/First Aid, 3-year Article 9/LCR-1034A).
4. Training method is identified per topic (in-person for skills like CPR and P.R.O.U.D.; in-person or online for knowledge topics).
5. Comprehension and competency are verified by quiz, skills demonstration, or supervisor observation, and documented.

6. Compliance monitoring: the Quality & Compliance Coordinator audits personnel files quarterly against §2 and the training matrix.
7. Violations result in immediate removal from schedules until cured, plus progressive discipline and systemic CAPs.
8. Oversight responsibility: Executive Director (policy), Training Coordinator (delivery/records), Quality & Compliance Coordinator (auditing).

5. Forms and references

Forms: DDD-1727A; LCR-1034A; FFI training matrix; Quick Reference Guide — Personnel Hiring & Training Requirements; AHCCCS DCW Training & Testing FAQs (reference).

References: QVA Service Requirements/Scope of Work & Standard Terms and Conditions; DDD Provider Policy Manual Ch. 20, 26, 41, 49, 61; A.A.C. R6-6-808, R6-6-809.C, R6-6-905, R6-6-906; A.R.S. §§ 36-558.01, 36-593.01.

FFI-09: Pre-Service Orientation

DOCUMENT NUMBER	FFI-09
POLICY OWNER	Training Coordinator
EFFECTIVE DATE	August 14, 2026
LAST REVIEWED	October 26, 2026
APPLIES TO	All FFI services (HAI/HID, GSE, ISE, ESA, PTE, TR1)
DDD PDT CHECKLIST	DDD-2051A § 2.25

1. Purpose

To ensure every staff member is oriented to each member's specific needs **before** delivering services, per the QVA, DDD form DD-097 (Pre-Service Orientation), and A.A.C. R6-6-808.E & F, R6-6-1512.C, and R6-6-1527.

2. Policy

- FFI uses the Division's Pre-Service Orientation form (**DDD-0097A**, and DDD-0097A-S where applicable). If an FFI form is ever substituted, it will contain at least the same information.
- Pre-service orientation is completed, signed, and filed **before** a staff member works with a DDD member, and prior authorization for the service is confirmed before service begins.

3. Procedures

1. **Completion before service.** The Program Manager or Employment Services Manager schedules the orientation as part of assignment; the schedule system blocks a DSP from being scheduled with a member until the signed orientation is on file.
2. **Content.** Orientation reviews the member's Planning Document and individualized health and safety needs, including behavior plans, medications, allergies, seizure protocols, adaptive equipment, communication preferences, emergency contacts, and back-up contacts.
3. **Signatures.** The orientation record is signed and dated by the staff member being oriented and by the supervisor or designee verifying the orientation is complete.
4. **Access for review.** Completed orientation documents are kept in the member's file and the staff training file; employees may review member-specific orientation information at any time through their supervisor (with confidentiality safeguards per FFI-01).
5. **Submission.** Completed and updated orientation documentation is sent to the member's DDD Support Coordinator, and updated whenever the member's needs materially change.
6. **Referrals and staff qualifications.** When a member is referred to FFI through the vendor call process or other means, the Program Manager or Employment Services Manager reviews the member's Planning Document before

accepting the referral and identifies the DSP who will serve the member. FFI accepts the referral only when the identified DSP has, or will complete before the first shift, the skills and training the Planning Document requires (for example, member-specific health tasks, behavior plan training, P.R.O.U.D., or communication needs). The skills match is documented in the member file.

7. **Matching and mutual agreement.** FFI matches DSPs to members based on member needs and preferences (skills, language, schedule, personality, gender preference for personal care) and confirms mutual agreement to serve — the member/responsible person and the DSP each confirm the match; either may request a change at any time.

4. Training, monitoring, and oversight

The Training Coordinator trains supervisors on conducting orientations. The Quality & Compliance Coordinator audits new assignments monthly to verify orientation preceded service and that documentation is complete and filed.

5. Compliance and corrective action

A DSP found serving a member without completed orientation is removed from the assignment until oriented; the scheduling failure is reviewed and corrected. Repeat violations result in progressive discipline.

6. Forms and references

Forms: DDD-0097A; DDD-0097A-S.

References: QVA Service Requirements/Scope of Work; DDD Provider Policy Manual Ch. 2; A.A.C. R6-6-808.E & F, R6-6-1512.C, R6-6-1527.

FFI-10: Workforce Development

DOCUMENT NUMBER	FFI-10
POLICY OWNER	Executive Director
EFFECTIVE DATE	August 14, 2026
LAST REVIEWED	October 26, 2026
APPLIES TO	All FFI services (HAI/HID, GSE, ISE, ESA, PTE, TR1)
DDD PDT CHECKLIST	DDD-2051A § 2.34

1. Purpose

To build and retain a qualified, stable direct support workforce, per DDD Provider Policy Manual Chapter 63, DDD Operations Policy Manual Chapter 407, and AHCCCS ACOM 407.

2. Policy

- FFI develops and maintains a written **Workforce Development Plan** (WDP), using the Division's Qualified Vendor Workforce Development Plan Tool Checklist (DDD-2280A FORFF) to develop the plan and to confirm, at each annual update, that every required component is included.
- The WDP contains the required components, including measurable goals for recruitment, retention, training, and career development of direct support staff.
- The WDP is **reviewed and updated at least annually**, including a written assessment of progress toward each goal.
- Workforce development activities are monitored throughout the year, not just at the annual review.
- FFI collects and analyzes workforce data — recruitment results, retention, turnover, and time-to-hire — quarterly, plus ad hoc data as needed to answer specific workforce questions.
- WDPs and their annual assessments are maintained on file and submitted to the Division upon request.

3. Plan components

FFI's WDP addresses, at minimum:

1. **Recruitment** — sourcing channels, hiring standards (FFI-08), and time-to-hire targets.
2. **Retention** — compensation review cadence, recognition, supervisor support, and stay/exit interviews.
3. **Training and career development** — onboarding and annual training calendar (FFI-08), competency pathways for DSPs (e.g., lead DSP, employment specialist), and tuition/certification support where feasible.
4. **Goals and measures** — each goal has a numeric target, data source, owner, and review date.

4. Monitoring and oversight

The Executive Director owns the WDP; the Training Coordinator maintains training-related data and the Quality & Compliance Coordinator compiles quarterly workforce metrics (turnover %, average time-to-hire, vacancy rate, training compliance %) for leadership review.

5. Compliance and corrective action

If quarterly data shows a goal off-track, the Executive Director documents an adjustment plan. Failure to maintain or produce the WDP is a compliance finding handled with an immediate CAP.

6. Forms and references

Forms: DDD-2280A FORFF Qualified Vendor Workforce Development Plan Tool Checklist; FFI Workforce Development Plan template; quarterly workforce metrics dashboard.

References: DDD Provider Policy Manual Ch. 63; DDD Operations Policy Manual Ch. 407; AHCCCS ACOM 407.



Part C — Service Delivery & Member Rights

FFI-11: Person-Centered Service Plan (PCSP)

DOCUMENT NUMBER	FFI-11
POLICY OWNER	Program Manager (Supported Living) / Employment Services Manager
EFFECTIVE DATE	August 14, 2026
LAST REVIEWED	October 26, 2026
APPLIES TO	All FFI services (HAI/HID, GSE, ISE, ESA, PTE, TR1)
DDD PDT CHECKLIST	DDD-2051A § 2.20

1. Purpose

To define FFI's role in the person-centered planning process and how FFI implements each member's Planning Document, per A.A.C. R6-6-805.C, E, F, G, I & J, R6-6-808.G & I, and R6-6-809.M.

2. FFI's role in the planning process

Responsibility by title:

- **Program Manager / Employment Services Manager:** reviews each member's Planning Document on receipt and before every planning meeting; requests changes when goals, supports, or risk information need updating; represents FFI at planning meetings; completes Planning Document assignments and team agreements accepted by FFI.
- **Quality & Compliance Coordinator:** ensures Progress Reports are completed and sent on the FFI-05 schedule to DDD, the member/representative, and the PCP.
- **DSPs:** implement the plan daily and raise concerns or observed changes to their supervisor.

3. Carrying out objectives and assignments

- Each member's goals are translated into teaching strategies (DDD-1783A format) within 20 business days of service initiation or goal change.
- **Documentation:** DSPs document each shift against plan objectives (services delivered, member response, data for each outcome).
- **Barriers to implementation:** when a barrier prevents carrying out an objective (member declines, health change, staffing, environmental issue), the DSP documents it the same day; the supervisor addresses it within 5 business days and, if unresolved, raises it to the member's planning team and Support Coordinator.

4. Required plan reviews

Before serving a member, and at every plan update, assigned staff review at minimum:

- the member's **Risk Assessment**, and
- the member's **Back-Up Plan** (emergency contacts and what to do when scheduled supports are unavailable),

along with behavior plans, health protocols, and communication preferences (see FFI-09).

5. Training, competency, and compliance systems

1. This policy and each member's Planning Document are reviewed with all direct care staff at least annually and whenever revised, with signed verification.
2. Comprehension and demonstrated competency are verified: supervisors observe each DSP implementing plan strategies within the first 30 days of assignment and at least annually.
3. Employee compliance with, and implementation of, the Planning Document is monitored through monthly documentation reviews and supervisor observation; findings feed FFI-06.

6. Compliance and corrective action

Failure to implement a member's plan (missed objectives, undocumented services, ignored barriers) results in retraining and progressive discipline; patterns trigger system CAPs.

7. Forms and references

Forms: DDD-1783A; DDD-1784A; FFI shift documentation; barrier log.

References: A.A.C. R6-6-805.C, E, F, G, I & J; R6-6-808.G & I; R6-6-809.M; DDD Operations Policy Manual Ch. 2000 (Planning Documents).

FFI-12: Communication with Responsible Parties

DOCUMENT NUMBER	FFI-12
POLICY OWNER	Program Manager (Supported Living) / Employment Services Manager
EFFECTIVE DATE	August 14, 2026
LAST REVIEWED	October 26, 2026
APPLIES TO	All FFI services (HAI/HID, GSE, ISE, ESA, PTE, TR1)
DDD PDT CHECKLIST	DDD-2051A § 2.05

1. Purpose

To set standards for respectful, lawful, and responsive communication with members and their responsible parties, per the QVA, DDD Provider Policy Manual Chapter 2, Operations Policy Manual Chapter 1001A, A.A.C. R6-6-108.C & H, R6-6-804.8, R6-6-805.B, R6-6-806.K, R6-6-808.G & I, R6-6-809.G & H, and A.R.S. § 36-551.01.P & Q.

2. Policy

- **Professional standards.** All staff communicate with members and responsible parties professionally and courteously — respectful tone, plain language, honoring the member's dignity, and timely responses (voicemail/email returned within one business day).
- **HIPAA compliance.** Information about a member is shared with family members or others **only** with a release of information signed by the member/guardian (FFI-01), except where law requires or permits disclosure. Staff verify guardianship/representative status before sharing.
- **Preferred methods.** At intake, FFI determines and documents each member's guardian/family preferred communication methods and frequency (phone, email, text, written progress reports, meetings) and honors them. Preferences are reconfirmed at least annually and at each planning meeting.
- **Emergency notification.** In an emergency involving the member, the responsible party is notified as soon as the member is safe — the same day for serious incidents, and within the timeframes in FFI-02 for all incident types. After-hours notifications are made by the On-Call Supervisor.

3. Procedures

1. Intake packet includes the Communication Preferences form; preferences are entered into the member record and visible to assigned staff.
2. Routine communications (schedule changes, progress updates) come from the assigned supervisor; DSPs refer questions beyond their role to the supervisor.
3. All substantive communications with responsible parties are documented in the member record (date, method, topic, outcome).

4. Interpretation/translation is provided at no cost when needed (FFI-13).

4. Training, monitoring, and oversight

Professional communication and this policy are covered at onboarding and annually. Supervisors monitor through documentation review and family feedback (FFI-14); complaints about communication are handled per FFI-06 §3.

5. Compliance and corrective action

Discourteous or unauthorized communication results in coaching, retraining, and progressive discipline; unauthorized PHI disclosure is handled under FFI-01.

6. Forms and references

Forms: FFI Communication Preferences form; contact log; Release of Information (FFI-01).

References: QVA Background, Service Requirements, Service Specifications; DDD Provider Policy Manual Ch. 2; Operations Policy Manual Ch. 1001A; A.A.C. R6-6-108.C & H, R6-6-804.8, R6-6-805.B, R6-6-806.K, R6-6-808.G & I, R6-6-809.G & H; A.R.S. § 36-551.01.P & Q.

FFI-13: Cultural Competency Plan

DOCUMENT NUMBER	FFI-13
POLICY OWNER	Executive Director
EFFECTIVE DATE	August 14, 2026
LAST REVIEWED	October 26, 2026
APPLIES TO	All FFI services (HAI/HID, GSE, ISE, ESA, PTE, TR1)
DDD PDT CHECKLIST	DDD-2051A § 2.07

1. Purpose

To ensure every member receives services that respect their language, culture, religion, and identity, per the QVA and DDD Provider Policy Manual Chapter 26.

2. Policy and methods

1. **Preferred language.** FFI identifies each member's preferred language at intake and ensures services are delivered in it — primarily by recruiting and matching staff who can meet the language needs of the members served, and by using qualified interpretation when a match is not available.
2. **Interpretation services.** FFI provides qualified interpretation (including telephonic/video interpretation services and ASL interpreters) at **no cost to DDD members**.
3. **Written translation.** FFI translates member-facing written documents (plans, schedules, progress reports, notices) into the member's/responsible person's language at **no cost**, using qualified translation services.
4. **Staff training.** Cultural competency training is provided at hire and annually (Training Coordinator; in person or online), covering culturally responsive communication, disability culture, religious and dietary accommodations, and LEP obligations. Additional/ongoing training and coaching are provided when a member's specific cultural needs call for it (for example, before starting services with a member whose traditions staff are unfamiliar with).
5. **Community cultural participation.** FFI encourages and supports member participation in cultural events and communities of their choosing — supporting transportation planning, schedule accommodations, and skill-building goals tied to cultural life.
6. **Religious and spiritual needs.** FFI supports members' religious practice (attendance, dietary rules, observances) and never imposes staff beliefs on members.
7. **Gender identity.** FFI supports each member's gender identity and expression, including using the member's preferred name and gender pronouns, and honoring stated gender preferences for personal care staffing (FFI-19).
8. **Feedback.** FFI obtains feedback on cultural and individual preferences through the annual satisfaction survey (FFI-14), planning meetings, and the complaint process, and adjusts services accordingly.

3. Monitoring, compliance, and corrective action

The Executive Director owns the plan; the Quality & Compliance Coordinator reviews cultural accommodation requests, survey results, and complaints annually and drives improvements. Disrespect of a member's culture, language, religion, or identity results in retraining and progressive discipline.

4. Forms and references

Forms: Intake language/cultural preferences form; interpreter request log.

References: QVA Service Requirements & Standard Terms and Conditions; DDD Provider Policy Manual Ch. 26; Title VI of the Civil Rights Act (LEP); ADA.

FFI-14: Family Feedback & Satisfaction Survey

DOCUMENT NUMBER	FFI-14
POLICY OWNER	Quality & Compliance Coordinator
EFFECTIVE DATE	August 14, 2026
LAST REVIEWED	October 26, 2026
APPLIES TO	All FFI services (HAI/HID, GSE, ISE, ESA, PTE, TR1)
DDD PDT CHECKLIST	DDD-2051A § 2.19

1. Purpose

To gather, analyze, and act on member and family feedback about FFI services, per the QVA Service Requirements.

2. Policy

- FFI conducts a member/family satisfaction survey **at least annually** for every member served (paper, electronic, or verbal administration to match accessibility needs and language, FFI-13).
- Survey topics include: quality and reliability of services, staff professionalism, communication, progress toward goals, cultural respect, safety, and overall satisfaction, with space for open comments.

3. Procedures

1. The Quality & Compliance Coordinator administers the survey each year and compiles results.
2. **Tracking and trending:** results are scored, logged, and compared year-over-year and against complaint/incident data.
3. **Review:** the Executive Director and the Program Manager / Employment Services Manager review results with the Quality & Compliance Coordinator within 30 days of survey close.
4. **Feedback to stakeholders:** the Executive Director provides formal feedback (a written summary of results and planned improvements) to members, families, and staff annually; supervisors provide informal feedback in planning meetings and day-to-day contact.
5. **Measuring improvement:** each identified improvement gets a measurable indicator (for example, fewer communication complaints, higher on-time service rate, improved satisfaction score) tracked in the QM program (FFI-06) and reported in the next annual summary.

4. Compliance and corrective action

Low or declining results in any area trigger a documented improvement plan with an owner and due date. Failure to administer the annual survey is a QM finding corrected by immediate scheduling.

5. Forms and references

Forms: FFI Satisfaction Survey; annual results summary template.

References: QVA Service Requirements; FFI-06 Quality Management Plan.

FFI-15: Behavior Management

DOCUMENT NUMBER	FFI-15
POLICY OWNER	Program Manager (Supported Living)
EFFECTIVE DATE	August 14, 2026
LAST REVIEWED	October 26, 2026
APPLIES TO	All FFI services (HAI/HID, GSE, ISE, ESA, PTE, TR1)
DDD PDT CHECKLIST	DDD-2051A § 2.04

1. Purpose

To ensure behavior support is positive, person-centered, and compliant with Article 9 (A.A.C. R6-6-900 et seq.), the DDD Behavior Supports Manual (including Chapter 900 — Emergency Measures and Physical Management Techniques), Provider Policy Manual Chapter 66, and A.A.C. R6-6-1007, R6-6-1107, R6-6-808.G & I, R6-6-809.B.

2. Policy

- **Prevention first.** Staff are trained in prevention and positive behavior support as applicable to their assignments: recognizing cues, reinforcing appropriate behavior, redirection, environmental evaluation, consistency, clear communication, and offering choices.
- **Article 9 alignment.** All behavior-related practices, plans, and training follow Article 9 requirements; techniques prohibited by Article 9 and FFI-03 §3 are never used.
- **P.R.O.U.D. training.** Staff who will support a member with a behavior plan or documented unsafe behavior complete P.R.O.U.D. (Providing Respectful Opportunities and Understanding De-escalation), taught by a Division-certified P.R.O.U.D. instructor, before working with that member. All DSPs hired after July 31, 2026, and any DSP retrained in member intervention techniques as a corrective action, are trained in P.R.O.U.D.; staff holding current Prevention & Support certification complete P.R.O.U.D. by December 31, 2027. Certification is renewed on the schedule set by the Division and tracked by the Training Coordinator (FFI-08).
- **Emergency measures.** Any use of emergency measures is an incident: staff follow Behavior Supports Manual Chapter 900, ensure member safety, and report per FFI-02 (immediate verbal report; entry into the AHCCCS QM Portal within the FFI-02 timeframes).

3. Prohibited and restricted techniques

Prohibited techniques (A.A.C. R6-6-902; Behavior Supports Manual Chapter 200) are never used with any member, under any circumstances:

- seclusion (locked time-out rooms, or holding a door shut to prevent a member from leaving);
- overcorrection;

- noxious stimuli (offensive tastes, sounds, odors, touch, or sights);
- physical or mechanical restraint used as a consequence for behavior, and any lying (supine, prone, or side) restraint or floor-seated restraint that folds or presses the body forward;
- behavior-modifying medication given "as needed" (PRN), in doses that interfere with daily living, or without a behavior plan, except as allowed by R6-6-909; and
- any treatment prohibited by A.R.S. § 36-561(A).

Techniques that require Program Review Committee (PRC) approval (A.A.C. R6-6-903; Behavior Supports Manual Chapter 400) are used only after the behavior plan containing them has been approved by the PRC and reviewed by the Independent Oversight Committee (IOC), and the responsible person has consented:

- techniques that require the use of force (for example, forced compliance or forced exclusion time-out);
- response-cost programs;
- programs that might infringe on the member's rights (for example, exclusion from routine activities, restitution, earning basic necessities, or limits on community access or possessions);
- behavior-modifying medications (except in the member's own or family home); and
- protective devices used to prevent self-injury.

4. Behavior plan writer

The Program Manager is FFI's designated behavior plan coordinator. When a member's planning team determines a behavior plan is needed, the plan is written by a qualified behavior plan writer who meets the Behavior Supports Manual Chapter 800 requirements, either a professional contracted by FFI or the writer assigned through the member's planning team, and is written on the Division's Behavior Plan template. The writer develops the plan by:

1. observing the member in multiple environments, at different times, and during different activities, following the member's lead;
2. interviewing the people who spend the most time with the member, using formal or informal assessment tools;
3. reviewing the member's history, antecedent-behavior-consequence (ABC) data, and incident reports related to behavior;
4. defining the target behaviors, identifying their function, and identifying replacement behaviors and the positive strategies to teach them;
5. naming who will train implementing staff and who will monitor the plan; and
6. reviewing the draft with the planning team and the responsible person for consent before PRC submission.

5. Behavior Treatment Plans (BTPs)

1. **Timely PRC submission.** When a member needs a BTP that requires review (techniques requiring force, response cost, potential rights infringement, behavior-modifying medications, or protective devices), FFI participates with the planning team so the plan is submitted to the Program Review Committee (PRC) — and IOC where applicable — **within 90 days of placement or of the need being identified**. The Program Manager tracks submission dates.

2. **Implementation.** Assigned staff are trained on each member's BTP before implementing it; implementation is documented every shift.
3. **Effectiveness monitoring.** Behavior data is collected as specified in the BTP, graphed or summarized monthly, and reviewed by the Program Manager with the planning team; ineffective plans are returned to the team for revision.
4. **Tracking system (Behavior Supports Manual Chapter 800).** The Program Manager maintains a behavior plan tracking log for every member with a plan, recording the target behaviors and their monthly data, the PRC submission and approval dates, the plan expiration date, and the date of each monitoring observation. The plan monitor named by the planning team (never a staff member who implements the plan) observes implementation on site at least twice a month and documents whether the plan is implemented accurately and consistently, whether data collection is complete and accurate, the member's progress toward goals, and any barriers or needed revisions. Data is evaluated at least monthly to judge effectiveness; plans expiring within 60 days are flagged for renewal.

6. Behavior-modifying medications

For members using behavior-modifying medications, FFI's procedures include:

- **Quarterly medication reviews** documented with the components listed under A.A.C. R6-6-806.M (medication, dosage, purpose, effectiveness, side effects, and review outcome), coordinated with the prescriber and planning team;
- **Screening for Tardive Dyskinesia** on the schedule set by the prescriber/planning team, with results documented; and
- **Prohibitions on psychotropic medication misuse** — never used as punishment, for staff convenience, or as a substitute for services, and only with informed consent and PRC review as required.

7. Documentation of maladaptive behavior

Staff document maladaptive behavior objectively (antecedent–behavior–consequence, duration, intensity, intervention used, outcome) in the member record the same shift; patterns are raised to the planning team.

8. Training and verification

- DSPs complete positive behavior support training at hire and annually; P.R.O.U.D. (certified instructor) as described in §2 (FFI-08).
- Direct Care Workers' training on this policy's requirements is documented with signed, dated records.
- This policy is reviewed by all DCWs at hire, at least annually, and whenever revised, with signed verification tracked by the Training Coordinator.

9. Monitoring, compliance, and corrective action

The Program Manager reviews behavior documentation monthly; the Quality & Compliance Coordinator audits BTP submission timeliness, quarterly medication review documentation, and training verification quarterly. Use of a prohibited technique triggers immediate removal from member contact, incident reporting, investigation, and discipline up to termination.

10. Forms and references

Forms: FFI behavior data sheets; DDD Behavior Plan template; FFI behavior plan tracking log; quarterly medication review form; DDD-1258A Emergency Contact Plan (crisis); AHCCCS QM Portal IAD report (FFI-02).

References: A.A.C. R6-6-900 series (Article 9), R6-6-1007, R6-6-1107, R6-6-808.G & I, R6-6-809.B; DDD Behavior Supports Manual Ch. 200, 400, 700, 800, & 900; Provider Policy Manual Ch. 4, 5, 10, 23, 66.

FFI-16: Health & Medical Care

DOCUMENT NUMBER	FFI-16
POLICY OWNER	Program Manager (Supported Living)
EFFECTIVE DATE	August 14, 2026
LAST REVIEWED	October 26, 2026
APPLIES TO	All FFI services (HAI/HID, GSE, ISE, ESA, PTE, TR1)
DDD PDT CHECKLIST	DDD-2051A § 2.13

1. Purpose

To ensure members' routine and emergency health and medical needs are met, per the QVA, DDD Provider Policy Manual Chapters 5, 7, and 8, A.A.C. R6-6-806 (incl. C & E), R6-6-808.G & I, R6-6-1011, and A.R.S. § 36-557.Q.

2. Routine medical care

- FFI supports members to access routine care (primary care, dental, vision, specialists) as identified in their Planning Documents — scheduling support, transportation coordination, and accompaniment when authorized.
- **Consent:** routine medical care occurs with the documented consent of the member or responsible person; consent status and who may consent are verified at intake and documented in the member record.
- **Documentation:** FFI maintains documentation of all medical appointments, emergency room visits, lab work, professional consultations, and treatments it supports, in the member record.
- **Supported Living (residential) members:** FFI supports members to see their primary care provider for well checks **at least annually** and completes any additional medical follow-up the PCP orders. FFI provides results of all healthcare appointments to the guardian **at least monthly** (and immediately for significant findings), per the member's release preferences.

3. Emergency medical care

- **Non-licensed staff call 911 and perform CPR** until a licensed healthcare professional arrives — **even when an Advance Directive is in place**. FFI staff do not interpret or execute advance directives in the field.
- After the member is stabilized, staff follow FFI-02: notify the On-Call Supervisor, responsible party, Division, and document the event.

4. Training

All staff are trained (at hire and annually; Training Coordinator; in-person skills components for CPR/First Aid) in:

1. Detection of signs of injury and/or illness;

2. Recognizing changes in health status (baseline awareness for assigned members);
3. Infectious disease precautions and the notification process (exposure reporting, standard precautions);
4. Response to non-emergency conditions that require medical attention (when to call the PCP, nurse line, or responsible person);
5. Medical emergency procedures and rendering emergency care (911, CPR/First Aid, staying with the member); and
6. Non-skilled tasks in the Nursing Assessment: when the Division's Health Care Services Nursing Assessment for a member lists non-skilled tasks for direct care staff, each assigned staff member is trained on those tasks for that member, and competency is verified by the supervisor (or a licensed nurse where the assessment calls for one), before performing them. Training is documented in the staff training file and repeated whenever the Nursing Assessment changes.

5. Monitoring, compliance, and corrective action

Supervisors review health documentation monthly; the Quality & Compliance Coordinator audits annual well-check completion and guardian reporting quarterly for supported-living members. Failures (missed appointments without follow-up, unreported health changes) trigger retraining, CAPs, and discipline as warranted.

6. Forms and references

Forms: FFI medical appointment log; health change note; consent for routine care.

References: QVA Standard Terms and Conditions; DDD Provider Policy Manual Ch. 5, 7, 8; A.A.C. R6-6-806, R6-6-806.C & E, R6-6-808.G & I, R6-6-1011; A.R.S. § 36-557.Q.

FFI-17: Medication Administration

DOCUMENT NUMBER	FFI-17
POLICY OWNER	Program Manager (Supported Living)
EFFECTIVE DATE	August 14, 2026
LAST REVIEWED	October 26, 2026
APPLIES TO	Habilitation, Supported Living (HAI/HID)
DDD PDT CHECKLIST	DDD-2051A § 2.16

1. Purpose

To define whether and how FFI staff handle member medications in supported living settings, per A.A.C. R6-6-806.C, R6-6-807.A, R6-6-808.G & I, R6-6-809.C, R6-6-902.B, R6-6-903.A, R6-6-908.B, A.R.S. § 36-551.01.O and § 36-557.O, DDD Operations Policy Manual Chapter 6002, and DDD Medical Policy Manual Chapter 310-V.

2. Policy: who may administer

- FFI DSPs who have completed the Division-recognized Medication Administration training (and member-specific orientation) **may assist with and administer medications** to supported-living members as identified in the member's Planning Document. Staff without current training may not handle medications.
- FFI verifies each member's PCSP/Planning Document to confirm the member's access to their medications and the level of support required (independent self-administration, reminders, assistance, or full administration). In any licensed setting, all members have the same access — medications are either locked or unlocked consistently per setting policy and member plans.

3. Requirements when staff administer medications

1. **Training:** employee medication administration training is documented with completion and expiration dates; refreshers occur before expiration.
2. **Storage:** medications are stored securely in original labeled containers, locked when required by the setting/plan, separated per member, with proper temperature control.
3. **Error reporting:** medication errors (wrong member, drug, dose, route, time, or missed dose) are reported immediately to the supervisor, the family/guardian, and DDD (incident report per FFI-02), with medical advice sought as needed.
4. **Medication log:** a Medication Administration Record (MAR) tracks every administration — medication, dose, route, time, initials.
5. **Disposal:** expired or discontinued medications are documented and disposed of per pharmacy/DEA guidance (witnessed, logged).

6. **PRN and behavior-modifying medications:** "as needed" administrations are documented with reason, time, and outcome; behavior-modifying medications follow FFI-15 §6.

4. Physician orders

- **All medication requires written physician orders — including over-the-counter medication.**
- A verbal order from a medical practitioner must be followed by a written order **within 72 hours**.

5. Administration procedure (step-by-step)

1. Wash hands; prepare a clean surface (prevention of contamination).
2. Verify the **right member, right medication, right dose, right time, right route** against the MAR and label.
3. Administer per route-specific training (oral, topical, rectal, or other trained routes).
4. Document immediately on the MAR; never pre-sign.
5. Observe for and report adverse reactions.
6. **Errors:** report per §3.3; the Quality & Compliance Coordinator reviews, tracks, and trends all errors and drives corrective actions.
7. **Refills:** staff monitor supply, request refills at least 7 days before exhaustion, and maintain an adequate supply.
8. **Self-administration:** members self-administer when the Planning Document team approves in writing; criteria for self-administration and documentation requirements are recorded in the plan, and staff document required supports.

6. Monitoring, compliance, and corrective action

Supervisors audit MARs weekly; the Quality & Compliance Coordinator trends errors monthly. Untrained staff handling medications, falsified MARs, or repeated errors result in immediate removal from medication duties, retraining, and progressive discipline.

7. Forms and references

Forms: MAR; medication error report (entered in the AHCCCS QM Portal when reportable); medication disposal log; physician order file.

References: A.A.C. R6-6-806.C, R6-6-807.A, R6-6-808.G & I, R6-6-809.C, R6-6-902.B, R6-6-903.A, R6-6-908.B, R6-6-609, R6-6-1011.F, R6-6-1109.B; A.R.S. § 36-551.01.O, § 36-557.O; DDD Operations Policy Manual Ch. 6002; DDD Medical Policy Manual Ch. 310-V; R6-6-803.A.6.

FFI-18: Client Property & Funds Management

DOCUMENT NUMBER	FFI-18
POLICY OWNER	Executive Director
EFFECTIVE DATE	August 14, 2026
LAST REVIEWED	October 26, 2026
APPLIES TO	Habilitation, Supported Living (HAI/HID); applicable member funds handling in all services
DDD PDT CHECKLIST	DDD-2051A § 2.18

1. Purpose

To safeguard, account for, and (when lost through FFI fault) replace member property and funds, per the QVA, DDD Operations Policy Manual Chapters 1001-A and 4004, and A.A.C. R6-6-807.A and R6-6-809.I.

2. Policy

- FFI safeguards member property and funds in its care; members' money is theirs, used only for their benefit and per their choices.
- FFI accepts responsibility for **acceptance, retention, disposition, accounting for, and administration** of member funds it handles under a member's Planning Document.
- **Joint accounts between members and FFI (or staff) are prohibited.** Staff never borrow from, lend to, or commingle funds with members.
- Member funds are never loaned, given, or provided in any way to other members, provider staff, relatives, or friends.

3. Member fund transactions when the Division is payee

Funds received through the Division's Client Funds system are handled through the Qualified Vendor Administration Site (not directly to staff), and:

- **Excess funds** are not used for non-approved purchases. If disbursed funds exceed the cost of the approved purchase, the excess is returned to the member's account with a reconciliation statement accounting for purchases.
- All non-personal spending money disbursed from the member's account for goods or services is **verified within 30 days by an itemized receipt**.

4. Discretionary funds and security

- Parameters for FFI access to and oversight of a member's discretionary spending money are set in the Planning Document; staff document all transactions they assist with.

- Member funds held at a site are secured in **locked storage** (locked cash box within a locked cabinet), access limited to the supervisor on duty, counted at each shift change involving custody transfer.
- Member funds held by FFI do not exceed **$200** per member; amounts above $200 are returned to the Client Fund System (or the representative payee) promptly.

5. Ledgers and reporting

- A **separate ledger** is maintained for each member's funds featuring: running totals, source of funds, date received, all expenditures with purchase descriptions, and attached receipts.
- Ledgers are sent to DDD Client Funds **monthly, on or before the 15th** (late ledgers block release of additional client funds until received).
- Ledger records are retained a minimum of **seven years** (FFI-05).

6. Property inventory (residential)

FFI maintains an inventory of each supported-living member's significant belongings, updated as items are accrued or discarded, and sends the updated inventory to the member's representative and Support Coordinator as changes are made.

7. Lost or stolen funds/property

When member funds or property are discovered lost or stolen, the Program Manager:

1. notifies the member/responsible person and the Executive Director the same day;
2. reports the loss or theft to the Division by the close of the next business day following its discovery;
3. enters the incident into the AHCCCS QM Portal no later than two business days after the loss or theft occurred or FFI was notified of it (FFI-02); and
4. for missing or lost member funds or property valued at less than $1,000, completes the Division reporting and resolution steps within 10 days, as set by the Division Operations Policy Manual.

Lost or stolen funds or property are **replaced by FFI within 10 working days** where the loss resulted from FFI's handling. FFI documents resolution and any repayment.

8. Representative payees

When DDD is not the member's representative payee, FFI coordinates with the actual representative payee (family member, agency, or the member) for receipts, reporting, and reconciliation, and documents the arrangement in the member record.

9. Training, monitoring, compliance

Client funds training is completed before any staff member handles member money, and annually. The Quality & Compliance Coordinator reconciles ledgers monthly, audits receipts quarterly, and verifies inventory updates.

Violations (missing funds, undocumented transactions, prohibited transactions) trigger immediate investigation (FFI-02/FFI-04), discipline up to termination, restitution, and system CAPs.

10. Forms and references

Forms: FFI member funds ledger; property inventory; receipt file; funds count sheet.

References: QVA Standard Terms and Conditions & Service Specifications; DDD Operations Policy Manual Ch. 1001-A, 4004 (A–O); A.A.C. R6-6-807.A, R6-6-809.I.

FFI-19: Staff-to-Client Ratios

DOCUMENT NUMBER	FFI-19
POLICY OWNER	Program Manager (Supported Living) / Employment Services Manager
EFFECTIVE DATE	August 14, 2026
LAST REVIEWED	October 26, 2026
APPLIES TO	HAI/HID, GSE, ISE, ESA, PTE
DDD PDT CHECKLIST	DDD-2051A § 2.31

1. Purpose

To ensure staffing is always adequate and consistent with each member's needs and service specifications, per the QVA Service Requirements and Service Specifications.

2. Policy

FFI's staffing addresses all of the following:

1. **Adequate staffing consistent with member needs** — ratios are set from each member's Planning Document and the collective needs of members served together.
2. **Planned absenteeism** — approved leave is backfilled at the time it is approved.
3. **Unexpected absenteeism** — the On-Call Supervisor maintains a same-day replacement roster; shifts are never left uncovered where the plan requires support.
4. **Emergencies** — emergency staffing follows FFI-26; the site is staffed as appropriate for the immediate circumstance.
5. **Community activities** — ratios for outings are planned in advance based on member needs, environment, and risk assessments.
6. **Maintaining required levels** — schedules are built to required ratios; supervisors verify coverage daily and adjust immediately when attendance changes.
7. **Ratio maintenance procedure** — service-specific ratio limits are enforced by the scheduling system and supervisor review (e.g., ISE job coaching is never less than 1 job coach : 1 member; GSE groups are 2–4 members with a paid FFI staff member remaining at the job site at all times; PTE does not exceed 1 DSP : 4 members with intermittent 1:1 as needed).
8. **Enhanced ratios** — when a member needs a richer ratio than the standard authorization, the manager documents the need and requests Division approval.
9. **Prior Division approval** — enhanced ratios are implemented **only after** approval by the Division.
10. **Unsupervised time** — a member's use of unsupervised time follows the member's Planning Document. Unsupervised time is never used to meet staffing needs or for staff/vendor convenience.

11. **Training before serving** — all staff are trained and oriented to members' specialized needs before working with them (FFI-09).
12. **Member choice** — members' right to personal care provided by staff of their choosing, including gender preference, is honored in scheduling and matching.

3. Monitoring, compliance, and corrective action

Supervisors verify ratios daily; the Quality & Compliance Coordinator audits schedules against authorizations and plans monthly. Understaffed shifts are service gaps reported per FFI-06 §5 and corrected; repeated scheduling failures result in CAPs and discipline.

4. Forms and references

Forms: FFI schedule/roster; enhanced ratio request; coverage exception log.

References: QVA Service Requirements/Scope of Work & Service Specifications (per-service ratio rules); FFI-20 (Daily Staffing Schedule for HID settings).

FFI-20: Daily Staffing Schedule (IDLA/HID Settings)

DOCUMENT NUMBER	FFI-20
POLICY OWNER	Program Manager (Supported Living)
EFFECTIVE DATE	August 14, 2026
LAST REVIEWED	October 26, 2026
APPLIES TO	Habilitation, Supported Living — daily/IDLA settings (HID)
DDD PDT CHECKLIST	DDD-2051A § 2.08

1. Purpose

To govern development, submission, and maintenance of Daily Staffing Schedules for FFI's supported-living (IDLA/HID) settings, per DDD Provider Policy Manual Chapter 52.

2. Policy

1. FFI maintains staffing ratios determined by the **collective needs of all members at the site**.
2. FFI submits all Staffing Schedules to the Division for review and approval **5 days before** members move into an Expansion Home or IDLA (HID) setting or before all known/planned events, and **within 2 days after** unplanned events.
3. FFI submits a new schedule for changes in occupancy, capacity (with Network approval), address, behavior or medication status that results in a modification to the staffing range, or home closure.
4. FFI submits a new schedule for school/holiday breaks or an inability of members to attend a day or work program that will result in changes to the staffing range.
5. **Emergencies:** FFI staffs the home as appropriate for the immediate circumstance.
6. If an emergency changes the staffing range, FFI notifies the **Network Manager the next business day** and the **Support Coordinator as soon as possible**, but no later than the next business day.
7. Staffing schedules include: members' first and last names; identification of members who have Behavior Plans; each member's work/day program schedule; additional staffing support needs; specific details regarding members' staffing needs; and, when a change is requested, the reason for the schedule change.
8. FFI submits an IDLA staffing schedule **annually** for approval. When a temporary staffing schedule is used, a new staffing schedule is submitted when the temporary schedule ends.
9. FFI meets with Network to review daily habilitation staffing schedules annually (as applicable to group-home settings; FFI applies the same review to IDLA settings on request).
10. FFI submits all agreed-upon updates to Staffing Schedules to DDD **within 14 calendar days** following the annual review.

3. Procedures

The Program Manager owns schedule creation using the IDLA Planning Guide (DDD-1856A) and IDLA staffing schedule format, validates it against members' Planning Documents, and tracks submission and approval dates. Approved schedules are posted for staff and kept in the site records.

4. Monitoring, compliance, and corrective action

The Quality & Compliance Coordinator verifies quarterly that operating schedules match approved schedules and that all triggering events generated timely submissions. Deviations are corrected by immediate resubmission and CAP.

5. Forms and references

Forms: IDLA Planning Guide (DDD-1856A); IDLA Staffing Schedule; IDLA Assurances (DDD-1734A).

References: DDD Provider Policy Manual Ch. 52; QVA Service Specifications — Habilitation, Supported Living.

FFI-21: Shift Change Communication

DOCUMENT NUMBER	FFI-21
POLICY OWNER	Program Manager (Supported Living)
EFFECTIVE DATE	August 14, 2026
LAST REVIEWED	October 26, 2026
APPLIES TO	Habilitation, Supported Living (HAI/HID) — sites with staff shift transitions
DDD PDT CHECKLIST	DDD-2051A § 2.29

1. Purpose

To ensure member needs and status changes are reliably communicated between staff at shift changes, per A.A.C. R6-6-803.A, R6-6-806.L & M, R6-6-808.G & I, R6-6-809.G, and R6-6-906.B.

2. Policy and procedures

1. **Shift handoff.** Outgoing staff communicate to relieving staff, verbally and in the written shift log, any needs and/or changes for each member during the shift: health status, behavior events, medications given or due, appointments, mood, meals, and follow-ups needed.
2. **Communication system.** FFI uses a standing site communication log (bound or electronic) covering events and changes affecting members' lives; the system is defined across FFI's QM plan (FFI-06), policies, training curriculum, and forms.
3. **Monitoring standards.** FFI applies the residential monitoring standards applicable to its settings (Article 8-equivalent oversight for supported-living sites): supervisor site visits, log reviews, and documentation checks.
4. **Staff questions encouraged.** Relieving staff are empowered and expected to ask questions about changes in a member's status before the outgoing staff leaves; leaving before completing handoff is a policy violation.
5. **Significant events.** Significant events are reported internally (supervisor/On-Call) and externally (Division, guardian) in a timely manner per FFI-02 — including incidents, medication errors, and medication changes.
6. **Other providers.** The Program Manager coordinates with members' other service providers (day programs, employment services, behavioral health) so services collectively address the member's needs; exchanges follow FFI-01 consent requirements.
7. **Documentation.** Internal communication about members is documented in the site communication log and member records: date, time, staff, topic, and any action needed.

3. Training, monitoring, compliance

Handoff procedures are part of onboarding and site orientation. Supervisors review shift logs weekly; missed handoffs or undocumented changes result in coaching, retraining, and progressive discipline.

4. Forms and references

Forms: FFI shift communication log; handoff checklist.

References: QVA Service Specifications; A.A.C. R6-6-803.A, R6-6-806.L & M, R6-6-808.G & I, R6-6-809.G, R6-6-906.B.

FFI-22: Visitors & Visitation

DOCUMENT NUMBER	FFI-22
POLICY OWNER	Program Manager (Supported Living)
EFFECTIVE DATE	August 14, 2026
LAST REVIEWED	October 26, 2026
APPLIES TO	HAI/HID settings and FFI program sites
DDD PDT CHECKLIST	DDD-2051A § 2.33

1. Purpose

To support members' right to visitors while maintaining safety, per DDD Operations Policy Manual Chapter 1001-C, Eligibility Policy Manual Chapter 100, A.A.C. R6-6-108.H, R6-6-804, R6-6-1006.J, R6-6-1009.A, R6-6-1014, R6-6-1114, and A.R.S. § 36-551.01.

2. Policy

- **Members' rights.** Members living in supported-living settings may receive visitors of their choosing at times of their choosing. FFI does not restrict member visitation **unless** a specific, documented health or safety risk is identified in the member's Planning Document — and then only to the extent the plan specifies.
- **Staff visitors.** Staff personal visitors are not permitted at member homes or program sites while staff are on duty, except with advance supervisor approval for a work-related purpose. Staff visitors never assume any care duties and are never left alone with members.
- **Roommates/households.** In shared supported-living homes, visiting arrangements respect all members of the household; conflicts are resolved through house meetings and planning teams, not blanket bans.

3. Procedures

1. **Sign-in/sign-out logs** are maintained at FFI program sites (and at shared supported-living sites where the household agrees or a plan requires): visitor name, member visited, time in/out.
2. **Ratios maintained.** Required staffing ratios (FFI-19) are maintained while visitors are on site; visitors do not count toward staffing.
3. **Safety.** Staff may ask a visitor to leave when behavior threatens any member's health or safety, and document the event (incident report if applicable).

4. Training, monitoring, compliance

Visitation rights and procedures are covered in onboarding. Supervisors review visitor logs during site visits. Unjustified visitor restrictions are rights violations — corrected immediately, documented, and addressed with

retraining/discipline.

5. Forms and references

Forms: FFI visitor log.

References: DDD Operations Policy Manual Ch. 1001-C; Eligibility Policy Manual Ch. 100; A.A.C. R6-6-108.H, R6-6-804, R6-6-1006.J, R6-6-1009.A, R6-6-1014, R6-6-1114; A.R.S. § 36-551.01.

FFI-23: Drugs, Alcohol & Smoking

DOCUMENT NUMBER	FFI-23
POLICY OWNER	Executive Director
EFFECTIVE DATE	August 14, 2026
LAST REVIEWED	October 26, 2026
APPLIES TO	All FFI services (HAI/HID, GSE, ISE, ESA, PTE, TR1)
DDD PDT CHECKLIST	DDD-2051A § 2.10

1. Purpose

To balance members' adult rights regarding alcohol and tobacco with health, safety, and a smoke-free environment, and to set absolute rules for staff, per DDD Operations Policy Manual Chapters 1001A and 1001B, A.A.C. R6-6-809.E & F, R6-6-808.G–I, A.R.S. § 36-568.02, and the Federal Controlled Substances Act.

2. Member rights and parameters

- **Rights.** Adult members retain the right to make lawful choices about alcohol and tobacco. Restrictions may exist only through the member's Planning Document, based on documented health or safety needs (for example, medication interactions), consistent with Article 9 — this policy does not itself restrict member rights.
- **Documented permissions and parameters.** Any permissions and parameters for a member's use of alcohol/tobacco — in the community, the supported-living setting, or at day/working programs — are identified and documented in the member's planning documents, and staff follow them. Staff document observed use only as those plans require.
- **Storage and consumption.** In shared supported-living homes, member-owned alcohol is stored in the member's private space (or as house agreements set out); consumption respects housemates' rights. Staff-owned alcohol is never brought to, stored at, or consumed at any service setting.

3. Smoke-free environment

- Every member has the **right to live in a smoke-free environment**. Indoor smoking is not permitted at FFI service settings.
- Members and staff who use tobacco do so outdoors in designated areas, positioned so smoke does not enter living areas or affect other members. Staff smoke only on breaks, away from members.

4. Staff prohibitions

- Staff are prohibited from using or being impaired by alcohol or drugs **while on duty**; reporting to work impaired is grounds for immediate removal from duty and discipline.

- Staff never purchase or furnish alcohol, tobacco, or any controlled substance for a member outside documented plan parameters, and never use member funds for such purchases outside the member's documented, lawful choices.

5. Marijuana and controlled substances

Marijuana (including CBD derivatives classified as controlled) remains a Class 1 controlled substance under federal law; see FFI-24 (Medical Marijuana) for member medical-use handling. Illegal drug possession or use at any service setting is reported to the Executive Director and, as applicable, law enforcement and the Division.

6. Rights review

This policy has been reviewed for Article 9 compliance: it imposes no restriction on member rights beyond documented, plan-based parameters, and prioritizes the equal right of all members to a smoke-free home.

7. Training, monitoring, compliance, and corrective action

Covered in onboarding and annual refreshers. Supervisors monitor during site visits. Violations are addressed through a dual system: **disciplinary action** for the individual (up to termination, plus incident reporting where a member was affected) and **system-based corrective action** (procedure or environment changes) to prevent recurrence.

8. Forms and references

Forms: Planning Document parameters; AHCCCS QM Portal IAD report (FFI-02) where applicable.

References: DDD Operations Policy Manual Ch. 1001A, 1001B; A.A.C. R6-6-809.E & F, R6-6-808.G–I, R6-6-1004.03; A.R.S. § 36-568.02; Federal Controlled Substances Act (21 U.S.C. § 811); A.R.S. § 41-1001(17); A.A.C. R9-17-202, R9-17-204, R9-17-310.

FFI-24: Medical Marijuana

DOCUMENT NUMBER	FFI-24
POLICY OWNER	Executive Director
EFFECTIVE DATE	August 14, 2026
LAST REVIEWED	October 26, 2026
APPLIES TO	All FFI services (HAI/HID, GSE, ISE, ESA, PTE, TR1)
DDD PDT CHECKLIST	DDD-2051A § 2.17

1. Purpose

To set interim standards for members who hold a valid Arizona medical marijuana card, pending final Division guidance. *Note: the corresponding DDD-2051A checklist (2.17) is under review by the Division of Developmental Disabilities; FFI will align this policy with Division guidance when issued.*

2. Policy

- FFI recognizes lawful medical marijuana use by adult members holding a valid AMMA (A.R.S. Title 36, Chapter 28.1) registry card, as a member right subject to the same plan-based parameters as other substances (FFI-23) and to federal law constraints.
- **Staff never purchase, transport, administer, or handle marijuana for members.** Member self-administration follows the member's documented plan; storage in shared homes is secured in the member's private space, inaccessible to other members.
- Use is documented in the member's planning documents (prescribing/certifying provider, form, parameters), reviewed with the planning team, and coordinated with prescribers given medication interactions (FFI-15 §6, FFI-17).
- Staff use or possession at any service setting or while on duty is prohibited regardless of cardholder status.
- Smoked or vaporized use follows the smoke-free rules in FFI-23 §3.

3. Monitoring, compliance, and corrective action

The Program Manager verifies planning-document alignment for any member using medical marijuana; issues (unsecured storage, staff handling, effects on health/safety) are addressed through the planning team, incident reporting where applicable, and staff discipline where staff violated this policy.

4. Forms and references

References: A.R.S. Title 36, Chapter 28.1 (AMMA); Federal Controlled Substances Act (21 U.S.C. § 811); A.A.C. R9-17-202, R9-17-204, R9-17-310; DDD guidance (pending — under Division review).

FFI-25: Pool & Swimming Safety

DOCUMENT NUMBER	FFI-25
POLICY OWNER	Program Manager (Supported Living)
EFFECTIVE DATE	August 14, 2026
LAST REVIEWED	October 26, 2026
APPLIES TO	HAI/HID and community activities in all services
DDD PDT CHECKLIST	DDD-2051A § 2.24

1. Purpose

To keep members safe around water, per A.A.C. R6-6-806.S, R6-6-1009.A-1, R6-6-1011.L, R6-6-1109.A-1, R6-6-1111.L, R6-6-1505.A, and OLCR Life Safety Inspections (Article 7).

2. Policy

1. **Bodies of water covered.** This policy applies to all situations involving bodies of water as defined by A.A.C. Title 6, Chapter 18, Article 1 — including pools, splash pads, rivers, lakes, canals, fountains, hot tubs, and fish ponds — whether at a member's home, in the community, or on outings.
2. **Risk assessment first.** Before any activity near water, staff confirm the member's Planning Document permissions, swimming ability, seizure risk, and required supervision level.
3. **Certified/licensed settings.** For all settings that are certified or licensed:
    - For any licensed residential setting, pools are enclosed per ADHS rules (A.A.C. Title 9, Chapter 33) — self-closing, self-latching barriers meeting code;
    - **Prior consent** is obtained from the member/responsible person indicating limitations of use of, or proximity to, bodies of water by the member; and
    - Staff responsible for supervision during water activities are **trained in water safety, first aid, and CPR** (current certifications on file).
4. **Supervision.** Members are supervised around water per their plans; staff maintain line-of-sight supervision for members with water-risk indicators, and no member is left unattended near water where their plan requires supervision.

3. Training, monitoring, compliance

Water safety expectations are part of onboarding and site orientation; CPR/First Aid currency is tracked under FFI-08. Supervisors verify water-related consents before outings involving water. Violations (unauthorized water activity, lapsed supervision) are treated as serious safety violations with incident reporting and discipline.

4. Forms and references

Forms: Water activity consent (LCR-1036A HBPNA basis); outing plan/risk check.

References: A.A.C. R6-6-806.S, R6-6-1009.A-1, R6-6-1011.L, R6-6-1109.A-1, R6-6-1111.L, R6-6-1505.A; A.A.C. Title 6 Ch. 18 Art. 1; A.A.C. Title 9 Ch. 33 (ADHS); OLCR Life Safety Inspections — Article 7.



Part D — Operations & Safety

FFI-26: Contingency, Emergency & Pandemic Plan

DOCUMENT NUMBER	FFI-26
POLICY OWNER	Executive Director
EFFECTIVE DATE	August 14, 2026
LAST REVIEWED	October 26, 2026
APPLIES TO	All FFI services (HAI/HID, GSE, ISE, ESA, PTE, TR1)
DDD PDT CHECKLIST	DDD-2051A § 2.06 (incl. Pandemic Plan)

1. Purpose

To ensure member health, safety, and service continuity through any emergency — staffing loss, facility loss, utility/system failure, natural disaster, or pandemic — per the QVA (Contingency Plan, I; Standard Terms and Conditions) and DDD Provider Policy Manual Chapter 36 (Fire Safety).

2. Health and safety in an emergency

The plan's first priority is the member: staff on duty secure member safety (evacuate, shelter, render aid, call 911 as needed), then notify the On-Call Supervisor at (602) 663-1664, who activates this plan and required notifications (FFI-02, FFI-12).

Notification to the Division. When an emergency affects members or FFI's ability to deliver services, the Executive Director (or designee) notifies the Division the same day:

- **Provider Network Support** — for all services, FFI reports the emergency, the members and services affected, and the steps FFI is taking to Provider Network Support (providernetworksupport@azdes.gov); and
- **Support Coordinator** — the Program Manager or Employment Services Manager notifies each affected member's DDD Support Coordinator, including any change in where or how services will be delivered.

Each notification is documented (date, time, person contacted, information given), and the Division and Support Coordinators are updated as the situation changes and when normal operations resume.

3. Workforce contingency

- **Key succession and performance planning:** if there is a sudden significant decrease in FFI's workforce (including loss of a key leader), pre-designated successors act in each role — Executive Director duties fall to the Program Manager; Program Manager duties to the Employment Services Manager (and vice versa) — until permanent coverage is in place.
- **Alternative service methods:** service continuity options include redeploying administrative staff with current DSP qualifications, adjusting schedules to cover plan-critical supports first, cross-agency staffing agreements, and (where appropriate and authorized) remote support check-ins.

- **Contact list and org chart:** FFI maintains an up-to-date contact list of all Qualified Vendor contacts by title, with an organizational chart, reviewed quarterly.
- **Staff call chart (phone tree):** a phone tree is maintained for emergencies; the On-Call Supervisor initiates it; each branch confirms completion back up the tree.

4. Facility and systems contingencies

- **Loss of facilities/sites:** if the FFI office or a service site becomes unusable, operations relocate to a pre-identified alternate location; members' services continue in their homes and communities wherever possible.
- **Electronic/telephone failure at the primary place of business:** calls forward to the On-Call cell number; leadership uses a group messaging channel as backup; the published number (602) 663-1664 remains reachable.
- **Loss of computer systems/records:** electronic records are backed up (FFI-05 §5); restoration is verified after any outage; paper fallback forms are staged for documentation continuity.
- **Facility evacuation plan:** each site maintains a posted evacuation route and assembly point; drills are conducted and documented; staff are trained at site orientation (fire safety per Provider Manual Ch. 36).
- **Shelter-in-place plan:** each supported-living site maintains supplies to shelter in place with adequate staffing levels, food, water, prescribed medications, and equipment meeting members' needs for **not less than 3 days** and for the duration of the emergency/disaster; supplies are inventoried quarterly.

5. Pandemic plan

During a declared pandemic or infectious disease emergency, FFI will:

1. Follow ADHS/CDC and Division guidance (including any DDD Qualified Vendor pandemic FAQs);
2. Implement infection control: screening, PPE per guidance, hygiene protocols, and enhanced cleaning at all sites;
3. Prioritize continuity of plan-critical services; document and report any service gaps to the Division;
4. Use the workforce contingency measures in §3 for staffing shortages, with member-preference and health-risk-based prioritization;
5. Communicate status changes to members, families, and the Division promptly (FFI-12); and
6. Track staff/member exposures and cases, reporting as required by public health authorities and the Division.

6. Training, monitoring, compliance

All staff are trained on this plan at hire and annually; site-specific evacuation/shelter training occurs at site orientation. The Executive Director tests the phone tree and reviews the full plan annually and after every activation (lessons-learned review feeding FFI-06). Failures to follow the plan are reviewed for retraining, discipline, and plan revision.

7. Forms and references

Forms: FFI contact list & org chart; phone tree; site evacuation plan; shelter-in-place inventory checklist; DDD-1258A Emergency Contact Plan (member crisis plans).

References: QVA Contingency Plan (I) & Standard Terms and Conditions; DDD Provider Policy Manual Ch. 36; Division Contingency/Pandemic Planning guidance.

FFI-27: Transportation

DOCUMENT NUMBER	FFI-27
POLICY OWNER	Employment Services Manager
EFFECTIVE DATE	August 14, 2026
LAST REVIEWED	October 26, 2026
APPLIES TO	TR1 (Employment-Related Transportation); member transport within HAI/HID, GSE, ISE, PTE
DDD PDT CHECKLIST	DDD-2051A § 2.32

1. Purpose

To ensure safe, reliable member transportation, per the QVA Service Specifications (Transportation), DDD Provider Policy Manual Chapters 38 and 49, DDD Medical Policy Manual Chapters 900 and 1250-H, and A.A.C. R6-6-1006.H, R6-6-1012, R6-6-1112, R6-6-1512.H.

2. Section A — operations

1. **Communication:** each vehicle used for member transport is equipped with a two-way radio or a cellular phone.
2. **Maintenance records:** vehicle maintenance records and safety inspection files are maintained and tracked for every vehicle used to transport members.
3. **Vehicle source:** FFI identifies, per assignment, whether employees drive FFI vehicles or their own vehicles. When personal vehicles are used, the same safety, insurance, and maintenance-record standards apply.
4. **Driver files:** each driver's license, vehicle registration, and car insurance are kept on file and verified at hire and annually.
5. **Member information:** drivers are provided with the health and safety information of all members being transported (seizure protocols, behavior considerations, communication needs, emergency contacts) before transport (FFI-09).
6. **Wheelchair transport:** members in wheelchairs are transported only in vehicles equipped with floor-mounted seat belts, shoulder straps, and wheelchair lock-downs or comparable safety equipment; staff are trained on securement before transporting.

3. Vehicle requirements

All vehicles used to transport members must have:

- Functional seat belts (all seats fastened to the vehicle body; members properly seated and belted);
- Functional wheelchair mounts/lifts (if applicable);
- Age-, size-, and weight-appropriate car seats available when needed to transport children;
- A system for communication in case of emergency (§2.1);

- A end-of-route check system ensuring all personal belongings are removed;
- A end-of-route check system ensuring **all members have exited the vehicle** (walk-through check, documented);
- Current valid registration and license plates;
- Operational heating and air conditioning;
- A First Aid kit; and
- The contractually required level of liability insurance (State of AZ Risk Management minimums).

4. Maintenance

For all vehicles used to transport DDD members — contracted and/or personal — FFI maintains records of normal/regular service and maintenance on file (oil changes, brakes, tires, inspections), reviewed by the Employment Services Manager quarterly.

5. TR1-specific requirements

For Transportation (TR1) service, FFI additionally:

1. Maintains a **back-up plan** activated whenever a scheduled driver or vehicle is late by 20 minutes or more (substitute driver/vehicle dispatch and member/family notification);
2. Keeps **logs of each member's pick-up and drop-off**, including time picked up from home, time dropped off at program, time departed program, and time dropped off at home;
3. Maintains on file a **daily transportation log** of services delivered to each member; and
4. Ensures availability of age-appropriate car seats (if applicable).

FFI provides coordinated, reliable transportation so members arrive on time for scheduled activities and are picked up upon completion.

6. Training, monitoring, compliance

Drivers complete responsible driving training (Provider Manual Ch. 49) and vehicle/securement orientation before transporting members, with annual refreshers. The Employment Services Manager audits driver files, vehicle checklists, and TR1 logs monthly. Accidents or member injuries during transport are incidents (FFI-02). Violations (unauthorized drivers, missing restraints, falsified logs) result in immediate removal from driving duties and progressive discipline.

7. Forms and references

Forms: FFI vehicle inspection checklist; daily transportation log; pick-up/drop-off log; driver file checklist.

References: QVA Service Specifications — Transportation; DDD Provider Policy Manual Ch. 38, 49, 51; DDD Medical Policy Manual Ch. 900, 1250-H; A.A.C. R6-6-1006.H, R6-6-1012, R6-6-1112, R6-6-1512.H.

Part E — Accessibility & Civil Rights

FFI-28: Americans with Disabilities Act (ADA)

DOCUMENT NUMBER	FFI-28
POLICY OWNER	Executive Director
EFFECTIVE DATE	October 26, 2026
LAST REVIEWED	October 26, 2026
APPLIES TO	All FFI services (HAI/HID, GSE, ISE, ESA, PTE, TR1)
DDD PDT CHECKLIST	DDD-2051A § 2.01

1. Purpose

To ensure members and their Responsible Persons can fully access FFI's services, programs, and communications, and can request reasonable modifications when a disability makes that necessary, per Title II of the ADA (28 CFR Part 35), A.R.S. § 41-1492.06(B), A.R.S. § 11-1024, and DDD Provider Policy Manual Chapter 24 (Americans with Disabilities Act).

2. Policy

- FFI does not discriminate on the basis of disability and makes reasonable modifications to its policies, practices, and procedures when needed to avoid discrimination, unless the modification would fundamentally alter the nature of the service or create an undue administrative or financial burden.
- FFI provides auxiliary aids and services, at no cost to the member, when needed for effective communication with members and Responsible Persons who have communication disabilities.
- The Executive Director is FFI's ADA Coordinator and is responsible for receiving, evaluating, deciding, and documenting ADA requests.

3. Notifying Responsible Persons how to request a modification

At intake, at each annual planning meeting, and whenever asked, FFI gives the member and Responsible Person a written notice (in their preferred language and format) explaining:

1. their right to request a reasonable modification or an auxiliary aid or service;
2. how to make a request: verbally or in writing to any FFI staff member, or directly to the ADA Coordinator at (602) 663-1664 or foundationforindependence@gmail.com;
3. how FFI evaluates requests and the timeframe for a decision; and
4. how to ask the Division for help, including the DDD ADA Requests Unit (DDDADArequests@azdes.gov).

The notice is also posted at FFI's office and at each supported-living site, and a copy of the signed acknowledgment is kept in the member record.

4. Evaluating requests

1. **Receipt.** Any staff member who receives a request (verbal or written) records it the same day on the FFI ADA Request Log and forwards it to the ADA Coordinator.
2. **Interactive discussion.** Within 5 business days, the ADA Coordinator contacts the member or Responsible Person to understand the barrier, the modification requested, and possible alternatives. Documentation of the disability is requested only when the need is not obvious, and only as much as is necessary.
3. **Evaluation.** The ADA Coordinator considers whether the modification is needed because of the member's disability, whether it is effective, and whether it would fundamentally alter the service or create an undue administrative or financial burden. When more than one effective option exists, FFI considers the member's preference first.
4. **Timeframe.** A decision is made within 15 business days of receipt; urgent health or safety requests are decided as quickly as possible, and interim measures are put in place while a request is pending.

5. Written determinations and records

- The ADA Coordinator sends the member and Responsible Person a written determination stating whether the request is granted, granted with an alternative, or elevated to the Division, the reasons for the decision, how the modification will be carried out, and how to ask for reconsideration or contact the DDD ADA Requests Unit.
- The request, notes from the interactive discussion, any supporting documents, and the written determination are kept in the member record and in FFI's ADA file for at least seven years (FFI-05). The ADA Request Log tracks each request from receipt to resolution.

6. Auxiliary aids and services

FFI informs members and families, in the notice in §3 and during orientation, that auxiliary aids and services are available when needed for effective communication, such as qualified sign language interpreters, assistive listening devices, large print, Braille, audio or electronic formats, plain-language materials, picture- or symbol-based communication supports, and extra time for communication. Interpretation and translation are coordinated with FFI-13.

7. Accessibility features

FFI informs members, at intake and in its member information materials, about the accessibility features it offers, including accessible locations for meetings, transportation planned around the member's mobility needs (FFI-27), documents available in alternative formats, and communication by phone, text, email, or video according to the member's needs.

8. Elevating requests to the Division's Title II ADA Unit

When the ADA Coordinator determines that a requested modification would fundamentally alter the nature of the program, service, or activity, or would create an undue administrative or financial burden, FFI does not deny the request on its own. Within 2 business days of that determination, the ADA Coordinator elevates the request to the Division's Title II ADA Unit through the DDD ADA Requests Unit (DDDADArequests@azdes.gov), notifies the

member's Support Coordinator, and informs the member and Responsible Person in writing that the request has been elevated. FFI follows the Division's determination and keeps it with the request record.

9. Training

All staff are trained on the ADA at hire and annually by the Training Coordinator (in person or online), covering members' rights under the ADA, how to recognize and record a modification request, auxiliary aids and services, respectful disability etiquette, and this policy's procedures. Completion and comprehension are documented in the staff training file (FFI-08).

10. Monitoring, compliance, and corrective action

The Quality & Compliance Coordinator reviews the ADA Request Log quarterly for timeliness and complete documentation and reports trends to the Executive Director as part of the Quality Management Plan (FFI-06). Failure to record or forward a request, or to follow this policy, results in retraining and progressive discipline; systemic issues result in a corrective action plan.

11. Forms and references

Forms: FFI ADA Rights and Request Notice (with member acknowledgment); FFI ADA Request Log; FFI ADA Written Determination letter.

References: 28 CFR Part 35 (Title II), Nondiscrimination on the Basis of Disability in State and Local Government Services; A.R.S. § 41-1492.06(B); A.R.S. § 11-1024; DDD Provider Policy Manual Chapter 24, Americans with Disabilities Act.